THE PRESIDENTIAL ADDRESS

DELIVERED

BY

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Ayurvedacharya
(Madras)

AT

THE 6th DRAVIDA VAIDYA CONFERENCE

HELD AT

TRICHINOPOLY
(28th May, 1934)



(TRANSLATED INTO ENGLISH BY S USHA)

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6TH DRĀVIDA VAIDYA CONFERENCE

TIRUCCIRĀPPALLI (28.5.34)

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ĀYURVEDA BHŪSAṆAM PAṆḌITA M DURAISVĀMI AIYANGĀR

ĀYURVEDĀCĀRYA

मध्ये मरुद्वृधाद्वीपं लोकातङ्कनिवृत्तये | धन्वन्तरिं पुरस्कृत्य स्थितं रङ्गेशमाश्रये ||

madhye marudvṛdhādvīpaṁ lokātaṅkanivṛttaye |
dhanvantariṁ puraskṛtya sthitaṁ raṅgeśamāśraye ||

"Seeking refuge in Bhagavān Ranganātha, the Lord of Śrīrangam, which is an island formed by the river Kāverī, I pay homage to Dhanvantari for removing the difficulties of people."

अयं भिषग्भ्यः पूर्वेभ्यः सादरं क्रियतेऽञ्जलिः | ये लोके वैद्यविद्यार्कस्याभवन्नुदयाद्रयः ||

ayaṁ bhiṣagbhyaḥ pūrvebhyassādaraṁ kriyate'ñjaliḥ |
ye loke vaidyavidyārkasyābhavannudayādrayaḥ ||

"I pay my respectful obeisance to the earlier physicians, who became the eastern mountains, as it were, for the rising sun of medical science (*viz*. Āyurveda) in this world."

भारतं विहतबाधमेधतां भारतीयमपि पातु भेषजम् |
शास्त्ररूढमतयोऽगदङ्करास्सन्तु शान्तकलहास्सुखैषिणः ||

bhāratam vihatabādhamedhatām bhāratīyamapi pātu bheṣajam |
śāstrarūḍhamatayo'gadaṅkarāssantu śāntakalahāssukhaiṣiṇaḥ ||

"Let (the country) Bhārata progress without any obstacle; also, may medicine protect the citizen! May the pharmacists, who are learned in the science of traditional medicine, be free from strife and be seekers of happiness (for themselves and for the people)!"

(Another meaning for the second line is: "May the pharmacists be experts in the science of traditional medicine, be free from strife and be seekers of all-round happiness!")

Foreword

Friends, while there are many renowned intellectuals in our country, members of this assembly have shown their trust in me by conferring on me the honour of delivering the Presidential Address, for which I am indebted. It will be proper to mention in this foreword the special points about Tiruchi, which is the venue of this conference and which, these days, is akin to the capital city of Tamilnadu. It will not be an exaggeration to say that this city can be called the crown of Tamilnadu due to the following reasons:

- After Chennai, Tiruchi has the highest number of cultured people.
- It has the office of the railways, whose tracks branch out in several directions.
- It boasts of renowned educational institutions, staffed by stalwarts in arts.
- It is also the home of prosperous tradesmen.
- It is like a crest among all the temples dedicated to Śiva and Vis n u.
- It is near the holy river Kāverī, which flows as two rivers (Kāverī, which is the southern branch, and Kol l id am, which is the northern branch).
- It is the abode of the sacred temple of Tāyumān ār (Śiva).

While Śrī Ranganātha's bestows health and happiness on the people with His endless compassion – in the form of the river Kāverī – He has also arranged for Bhagavān Dhanvantari, who is but His partial manifestation (*am śāvatāra*) to be present permanently in His temple to heal the people if they become unwell. Where else can one come across such a wondrous sight?

I would also like to remind the audience that Deśabhūs an am R K Prasanna Venkateswara Rao, an ardent devotee of Bhagavān and my very close friend, established an Ayurvedic hospital here in Śrīrangam twenty years ago, out of an extraordinary interest in the subject. It was inaugurated by my late preceptor, Vaidyaratnam Pandit D Gopalacharlu.

At least a few of you may recall that, after completing my studies at the Ayurvedic College in Chennai and acquiring eligibility, I started my practice in this city in 1908. Through this Foreword, I would like to point out that, considering the above points, it is only proper that this Ayurvedic conference is held in this town – which has been my residence and work-place for the last three years and is hence close to me – and that, in deference to the support of the assembled members, I have accepted this post of President.

Condolence to the Departed

Before continuing on the topics mentioned above, I would like to follow the practice of offering condolences on the demise of prominent physicians. I express my sorrow on the untimely passing away of Dr. S Rangachari, a staunch advocate and a steadfast practitioner of Allopathy. Though he did not evince interest in Ayurveda, he was friendly and polite towards me during our occasional meetings. In his practice, he had unreservedly and compassionately treated thousands of people. Just as Pandit D Gopalacharlu was the 'soul' of Ayurveda in Chennai, Dr. Rangachari too was an incomparable physician in Allopathy. His extensive knowledge in his field and unflagging enthusiasm were the reasons for his fame and people's unshakeable trust in him. Our physicians too should follow his example through their sincere efforts. Instead of feeling jealous at his success, if we are motivated by his hard work, we can achieve a similar success (*As॰t॰āngahr॰dayam – Sūtrasthānam* 2.25: '*hetāvīrs॰yet phale na tu*'). May his soul rest in peace!

Conference

The encouraging words of the members of this assembly have enthused me. The *Caraka-sam॰hitā* states that our ancestors, *viz*. the great sages, used to gather in hundreds in places like Himālaya and Naimiśāran॰yam to debate on the scriptures and decide several matters for the welfare of the society.

विघ्नीभूता यदा रोगाः प्रादुर्भूताः शरीरिणाम् | तपोपवासाध्ययनब्रह्मचर्यव्रतायुषाम् ||
तदा भूतेष्वनुक्रोशं पुरस्कृत्य महर्षयः | समेताः पुण्यकर्माणः पार्श्वे हिमवतः शुभे ||
(*Carakasam॰hitā – Sūtrasthanam* 3-4)

vighnībhūtā yadā rogāh॰ prādurbhūtāh॰ śarīrin॰ām |
tapopavāsādhyayana-brahmacaryavratāyus॰ām ||
tadā bhūtes॰vanukrośam॰ puraskr॰tya mahars॰ayah॰ |
sametāh॰ pun॰yakarmān॰ah॰ pārśve himavatah॰ śubhe ||

"At the onset of diseases which were obstacles to penance, education, austerities and life-span of embodied souls (humans), our noble sages, out of compassion for all creatures, gathered at the foothills of the Himālayas to discuss remedies (for those diseases)."

On that occasion, great sages like Angiras, Jamadagni, Kāśyapa, Bhr॰gu, Agastya, Pulastya, Ātreya, Gautama, Asita, Vāmadeva, Āśvalāyana, etc. (the names of sixty such sages have been mentioned, together with the Vālakhilyas, the Vaikhānasas and several other sages who were repositories of sacred knowledge) assembled, discussed the situation and came to a decision. This has been stated in various chapters in the *Carakasam॰hitā*. Reading these, it is

apparent that our idea of gatherings and discussions today is not so great, after all. Though the difference between us and the sages – who were endowed with divine power – is like the difference between a goat and an elephant (‘*aja-gaja-antara*’), let us do our best to spread knowledge by following the footsteps of those great souls, just as a thread easily enters a gem after a hole has been drilled in the latter using diamond (diamond-tipped borer).

मणौ वज्रसमुत्कीर्णे सूत्रस्येवास्ति मे गतिः

(*Raghuvaṁśa* 1.4)

maṇau vajrasamutkīrṇe sūtrasyevāsti me gatiḥ

(My path is like that of a thread passing through a gem that has been previously perforated by diamond-drill.)

Āyurveda – The Basis for All Methods of Treatment

When we realise that, right from ancient time, our country, Bhārata, has shone as the fountainhead for many branches of science and arts, do our hearts not overflow with pride? Earlier, during a Sanskrit conference held in Chennai, I had mentioned that the word ‘*bhāratī*’ clearly describes how Bhārata has been the birthplace of all knowledge for several eons, nay, right from the time of creation. This statement was accepted by everyone in the audience. Even the historians of today declare that, while humans in other countries were wandering around naked, our country, free from any form of uncivilised behaviour, was illuminating the world with its knowledge. When the residents of those countries gradually learnt to think rationally and got accustomed to live in cities and towns, they came to our country, either as invaders or through other means, and observed our culture as well as our methods of teaching the different branches of knowledge. It is but naturally human that they attempted to acquire all these information and put them to use in their country. The early visitors to our country evinced immense interest in our medical science, which is essential to save lives and is, therefore, more valuable than dialectics and grammar, which require only oratorical skills to participate in a debate. There are historical records which state that some of them not only learnt many of the methods of treatment, but also took away some skilled physicians as well as valuable texts when they returned to their country.

In his book ‘*Studies in the Medicine of Ancient India*’, Dr. A F R Hoernle, M.A., Ph.D., C.I.E has written as follows[1]:

[1] The original passage from the book has been reproduced here.

> "Probably it will come as a surprise to many, as it did to myself, to discover the amount of anatomical knowledge which is disclosed in the works of the earliest medical writers of India. Its extent and accuracy are surprising, when we allow for their early age – probably the sixth century before Christ – and their peculiar methods of definition. In these circumstances the interesting question of the relation of the Medicine of the Indians to that of the Greeks naturally suggests itself. The possibility, at least, of a dependence of either on the other cannot well be denied, when we know as an historical fact that two Greek physicians, Ktesias, about 400 B.C., and Magasthenes about 300 B.C., visited, or resided in, Northern India."

I would like to present to this assembly the following passage from the book '*History of Medicine*', written by Dr. Max Neuburger[2]:

> "That Greek medicine adopted Indian medicaments and methods is evident from the literature. The contact between the two civilisations first became intimate through the march of Alexander, and continued unbroken throughout the reign of Diadochi and the Roman and Byzantine eras. Alexandria, Syria and Persia were the principal centres of intercourse; Indian physicians, means and methods of healing are frequently mentioned by Graeco-Roman and Byzantine authors, as well as many diseases, endemic in India, but previously unknown. During the rule of the Abbasides, the Indian physicians attained still greater repute in Persia, whereby Indian medicine became engrafted upon the Arabic, an effect which was hardly increased by the Arabic dominion over India. Indian influence, in the guise of Arabic medicine, was felt anew in the West. The apparent spontaneous appearance of Sicily in the fifteenth century of rhino-plastic surgery bespeaks a long period of previous Indo-Arabian influence. The plastic surgery of the nineteenth century was stimulated by the example of Indian methods; the first occasion being the news derived from India, that a man of the brick-makers' caste had, by means of a flap from the skin of the forehead, fashioned a substitute for the nose of a native."

(This type of treatment is described in the *Karn॰aroga-nāsāroga-pratis॰edhādhyāya* of the *Uttarasthānam* in the *As॰t॰āngahr॰dayam*.)

In his book '*A Short History of Aryan Medical Science*', His Highness, Sir Bhagvat Sinh Jee, Thakore Saheb of Gondal (the ruler of Gondal), has given several evidences and stated thus[3]:

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[2] The original passage from the book has been reproduced here.

[3] The original passage from the book has been reproduced here.

> "The great works of Charaka and Sushruta were translated into Arabic, under the patronage of Khalif Almansur, in the seventh century..... These translations, in their turn, were rendered into Latin. The Latin versions formed the basis of European medicine....."

Futile Dispute

While the western scholars have, on the basis of historical studies, proved that Āyurveda is the fountainhead for all medical systems in the world, a new name, 'Siddha-vaidya' has emerged in the southern part of India, with claims that this system of medicine is completely different from Āyurveda. That these assertions have started creating a schism among doctors and are therefore dangerous has been my contention from the very beginning. We claim that Āyurveda, with its aspects of (i) *tridos*ॅ*a* (the three humours, *viz*. *vāta*, *pitta* and *kapha*), (ii) *rasa-vīrya-vipāka-prabhāva* (the importance of taste, potent energy and the effects of foods), (iii) *nidānapañcaka* (the fivefold method of diagnosis in Āyurveda), (iv) the therapies of *vamana-virecana-snehana-svetana* (emesis, purgation, lubrication, perspiration – these are the methods to remove toxins from the body) is distinct from Allopathy, which does not have these features. But how can Āyurveda be different from the system mentioned in the Tamil medical books, when both of them are based on these same factors, such as *tridos*ॅ*a*, etc.? It is claimed that mineralogy and metallurgy is a special feature in the Tamil texts. But repeated studies of various Tamil medical texts have failed to provide details of any systematic classification of various minerals. If descriptions about the methods of preparing ashes (*bhasma*) and oxides (*sindūra*) of various minerals are said to be found in some places in a few texts, then how can the system itself be described as discrete (from Āyurveda)?

Besides, it is not as if medical treatises on mineralogy are not available in Sanskrit. The study of mineralogy is a separate branch of Āyurveda, with not just one, but several texts devoted to its exposition. Numerous independent works, such as the *Rasaratnākara*, the *Rasaratna-samuccaya*, the *Rasārn*ॅ*ava*, the *Rasahr*ॅ*dayam*, the *Rasamañjarī*, the *Rasatarangin*ॅ*ī*, the *Rasarājasundara*, the *Raseśvaratantra*, the *Rasaprakāśasudhākara*, the *Rasasanketakalikā*, the *Rasapaddhati*, the *Āyurvedaprakāśa*, the *Lohasarvasvam*, etc. have been written in Sanskrit by the *siddhas* (mystics), with many of these works still in print. Learned people will refute the irrational and separatist arguments that the system of Siddha medicine was established by eighteen *siddhas*, who wrote medical texts in Tamil. Many of those *siddhas* were scholars in Sanskrit; many have penned medical books in Sanskrit as well. The *siddhas* were not just eighteen in number. I have recently written and published a Tamil translation of Vāgbhaॅta's *Rasaratnasamuccaya*, so that our Tamil brethren, who do not know Sanskrit, can also benefit from it. In the beginning of that book, Vāgbhaॅta states that he composed this work after studying the various Sanskrit texts on mineralogy which had been written earlier by twenty-seven *siddhas*. All these books were composed in North India

and published in Devanāgari script by the local citizens. As the publishers have mentioned in their Forewords that the manuscripts of these books were sourced from Kashmir, Nepal and Punjab, there is no evidence that these texts were copied from Tamil works.

A study of the Sanskrit texts on mineralogy, setting aside any feeling of jealousy, will reveal many new aspects. Firstly, minerals are divided into various groups, such as *rasa* (minerals in general), *mahārasa* (precious minerals), *uparasa* (secondary minerals) *sādhāran*◌ॅ*arasa* (a group of eight metals inferior to *uparasa*), and *dhātu* (other natural sources of minerals and metals). They are also classified as *loha* (metals), *ratna* (gems) and *khanija* (ores). These details are found in the *Rasaratnasamuccaya* as well as other books on Āyurveda. The different varieties of each ore, together with their characteristics, have been thoroughly analysed only in these texts. For example, the metal *aya* (iron) is classified into three – *mun*◌ॅ*d*◌ॅ*a-loha* (cast iron), *tīks*◌ॅ*n*◌ॅ*a-loha* (carbon steel) and *kānta-loha* (soft iron). *Mun*◌ॅ*d*◌ॅ*a-loha* is further divided into three groups – *mr*◌ॅ*du*, *kun*◌ॅ*t*◌ॅ*ham* and *kad*◌ॅ*āram*. *Tīks*◌ॅ*n*◌ॅ*a-loha* is categorised into *kharam*, *sāram*, *hr*◌ॅ*nnālam*, *tārāvat*◌ॅ*t*◌ॅ*am*, *vājiram* and *kālam*. *Kānta-loha* is grouped into *bhrāmakam*, *cumbakam*, *kars*◌ॅ*akam*, *drāvakam* and *romakāntam*. Hence we can see that a single metal, *viz*. iron, has thirteen varieties, with each group having its own characteristics. Similarly, the mineral *tuvarī* (alum) is described thus in the *Rasaratnasamuccaya* (Chapter 3, verses 59 to 62).

सौराष्ट्र्यश्मनि सम्भूता मृत्स्ना सा तुवरी मता | वस्त्रेषु लिप्यते याऽसौ मञ्जिष्ठा रागबन्धिनी ||
फटकी फुल्लिका चेति द्वितीया परिकीर्तिता | ईषत्पीता गुरुः स्निग्धा पीतिका विषनाशिनी ||
व्रणकुष्ठहरा सर्वकुष्ठघ्नी च विशेषतः ||
निर्भारा शुभ्रवर्णा च स्निग्धा साम्लाऽपरा मता | सा फुल्लतुवरी प्रोक्ता लेपात्ताम्रं चरेदयः ||

saurās◌ॅ*t*◌ॅ*ryaśmani sambhūtā mr*◌ॅ*tsnā sā tuvarī matā* |
vastres◌ॅ*u lipyate ya'sau mañjis*◌ॅ*t*◌ॅ*hā rāgabandhinī* ||
phat◌ॅ*akī phullikā ceti dvitīyā parikīrtitā* |
īs◌ॅ*atpītā guruh*◌ॅ *snigdhā pītikā vis*◌ॅ*anāśinī* ||
vran◌ॅ*akus*◌ॅ*t*◌ॅ*haharā sarvakus*◌ॅ*t*◌ॅ*haghnī ca viśes*◌ॅ*atah*◌ॅ ||
nirbhārā śubhravarn◌ॅ*ā ca snigdhā sāmlā'parā matā* |
sā phullatuvarī proktā lepāttāmram◌ॅ *caredayah*◌ॅ ||

"*Tuvarī* is a kind of mud produced from a rock found in the Saurās◌ॅt◌ॅra region; when it comes in contact with cloth, it colours the fabric, just like *mañjis*◌ॅ*t*◌ॅ*ha* (*Rubia Cordifolia* or Indian Madder, whose roots produce a natural red dye). It is of two types – *phat*◌ॅ*akī* and *phullikā*. The first is slightly yellow, heavy and viscous. It destroys poison, heals wounds and is used to treat all types of leprosy. The other is lightweight, white in colour, glutinous and

sour to taste. This is *phullatuvarī* which, when smeared on iron, turns the latter into reddish copper."

Likewise, *uparasas* (secondary minerals) such as *kāsīsam* or *annabhedī* (ferrous sulphate), *tuttham* (copper sulphate), *tālakam* (arsenic sulphide), *manah*◌̃*śila* (red arsenic), *pās*◌̃*ān*◌̃*a* (white arsenic), *navasāram* (ammonium chloride) have also been described in the *Rasaratna-samuccaya* and other texts. Moreover, a remarkably comprehensive list of all the aspects of *rasatantra* (Āyurvedic chemisty) is given in the *Rasaratnasamuccaya*. Right from describing the manner in which a chemical laboratory should be set up, specifying the place for each apparatus to be kept in readiness, delineating many rare purificatory methods – like those for preparing *kajjalī*, *rasapanka*, *pis*◌̃*t*◌̃*i*, *hemakars*◌̃*an*◌̽*am*, *tārakars*◌̃*an*◌̃*am*, *piñjarī*, *nirvāpan*◌̃*am*, *ūnamam*, *bījam*, *satvapātanam*, etc., explaining the various *yantrams* (apparatus) such as *dolā-yantram*, *pātanā-yantram*, *dīpikā-yantram*, *d*◌̃*ekī-yantram*, *jāran*◌̃*ā-yantram*, *kacchapa-yantram*, *somānala-yantram*, *vālukā-yantram*, *ham*◌̐*sapāka-yantram*, *jala-yantram*, etc., which are used to refine the minerals, detailing the different varieties and uses of *mūs*◌̃*ās* (crucibles) like *vajramūs*◌̃*ā*, *yogamūs*◌̃*ā*, *gāramūs*◌̃*ā*, *varamūs*◌̃*ā*, *vid*◌̃*amūs*◌̃*ā*, *vr*◌̽*ntākamūs*◌̃*ā*, etc. to hold the minerals during the process, defining the forms and uses of the varied kinds of *kos*◌̤*t*◌̤*hikā* (ovens), *viz*. *pātāla-kos*◌̤*t*◌̤*hikā*, *angāra-kos*◌̤*t*◌̤*hikā*, *satvapātana-kos*◌̃*t*◌̤*hikā*, *vankanāla-kos*◌̤*t*◌̤*hikā*, etc., outlining the many methods of *put*◌̤*a* (incineration) like *mahāput*◌̤*a*, *gajaput*◌̤*a*, *varāhaput*◌̤*a*, *kukkut*◌̤*aput*◌̤*a, kapotaput*◌̤*a*, *bhūdharaput*◌̤*a*, etc. used to purify and increase the efficacy of the minerals, listing the eighteen *sam*◌̐*skāras* (procedures) of *svedanam*, *mardanam*, *mūrchanam*, *uddharan*◌̃*am*, *pātanam*, *nirodhanam*, etc. employed in further refining and strengthening mercury, specifying the *bandhas* (binding techniques) like *ārot*◌̤*a-bandha*, *hat*◌̤*ha-bandha*, *kriyāhīna-bandha*, *pis*◌̤*t*◌̤*ikā-bandha*, *kalka-bandha*, *bīja-bandha* and *druti-bandha*, which are used to control the mobility of mercury – together with hundreds of other technical details that cannot be fully described here – the contents of the Sanskrit works and their detailed expositions will definitely help the objective reader decide that these alone are the true and correct Siddha texts.

I repeat here what I have always maintained in the past – that the split of our medical brothers in Tamilnadu, as a consequence of their claims about the greatness and distinctness of their system merely on the basis of '*muppu*' (a combination of three salts, which is said to increase the effectiveness of a medicine), is improper. But despite monumental efforts, such as setting up a separate department for it, this combination is yet to be found. My opinion is that this '*muppu*' is a mixture of the three *ks*◌̃*āras* (alkalis), *viz*. *sarjaks*◌̃*āra* (sodium bicarbonate), *yavaks*◌̃*āra* (alkali extracted from barley) and *t*◌̤*ankan*◌̽*aks*◌̃*āra* (borax), which is used in the incineration of metals like iron. Whatever be my stand in this matter, there has been no one who has the grit to counter that using this term ('*muppu*') alone to contend that this system is different, to give it a new name and to form a separate department for it are merely political attempts and not based on the available textual evidence.

The numerous books on mineralogy – some of which I have listed here and some omitted –have all been written by the *siddhas*. The Sanskrit texts contain hundreds of concepts as well as all the aspects essential for chemistry that are not found in the Tamil works. The methods of treatment for all the diseases mentioned in the *Mādhavanidānam* have been discussed, in the same order, in the *Br*ndamādhavam, which is also called '*Siddhayoga*'. Thus, an analysis against the backdrop of the appropriate ancient texts provides sufficient reasons to describe Āyurvedic mineralogy and Āyurveda itself as 'Siddha-vaidya'. Preceptors like Punarvasu Ātreya, who taught medicine to Agniveśa and others, are also referred to as '*siddha-munis*'.

Despite proclamations about the existence of several works on Siddha-vaidya, no one has, in all these years, revealed the names of the authentic and academic Tamil books that are comparable to the *Carakasam*hitā and other (Sanskrit) texts. For want of proper text books, a mere compilation, said to be from various works, has been published; there is no information about the author (of this compilation). However, it is useless to indulge in endless, unreasonable arguments. Even conceding to the arguments of Siddha-vaidya practitioners and accepting their assertions that the Āyurvedic texts – discovered in Kashmir, Nepal and Ujjain in North India and also in China (where Bower's manuscript was found), which were written by Vāgbhat a and other Sanskrit physicians as well as by sages like Caraka and others, none of whom was even aware of the Tamil language – were based on the Tamil works found in South India, the fact still remains that there is no dissimilarity between the two systems. In that case, is it not futile to assert that these two medical systems are different? Medical systems that have the concept of *tridos a* as their basis are not different from one another.

Two books are said to be the prescribed texts for studying the Siddha medical system – the '*Jīvaraks āmr tam*' and the '*Ātmaraks āmr tam*'. Neither of them is written by a *siddha*, nor are they based on any Siddha medical text. Though based on Āyurvedic literature and containing information sourced from them, these two books are held to be Siddha texts merely because they have been compiled by Śrimān Muttaiya Pan d ita and others.

Why a new name?

In fact, until 1914, all the doctors in Tamilnadu used to refer to themselves as traditional physicians or Āyurvedic physicians. Until then, no one had either seen or heard of the Siddha medical system, either in practice or in books. In 1914, Śrimān Maduraimuttu Mudaliyār, Pan d it Ānandam, Murugeśa Mudaliyār, Muttaiyā Pil l ai, Śivajñāna Yogi came together to form the '*Ten n indiya Āyurvēda Sangam*' ('South Indian Ayurveda Association'). Several persons who call themselves as 'Siddha doctors' were members of this

association. This fact is seen in the report (vol. 2, page 449) of the Usman Committee, which was commissioned in 1921 by the Government to study the indigenous systems of medicine. In his testimony, my friend, Śrīmān Murugeśa Mudaliyār (now 'Vaidyaratna'), has declared that the concept of *tridos*◌ा*a* is the basis for his system of medicine and that there is no specific difference between Āyurveda and Siddha, but has stated that the usage of *bhasma* and *sindūra* is unique to his Siddha method. This is what I have objected to in the beginning. Will the systems themselves have drastic differences merely due to the usage of *bhasma* and *sindūra*? The Sanskrit texts on mineralogy that I had listed earlier describe numerous methods – some easy and some difficult – that use *bhasma* and *sindūra*. I am aware of some of these physicians calling their system of treatment as 'Agattiya-vaidya', others referring to it as 'Tamil-vaidya', still others labelling it 'Tamil Siddha-vaidya' and yet others arguing that a different name should be given to this system. The terms 'Siddha-vaidya' or 'Tamil-vaidya' or 'Agattiya-vaidya' cannot be seen in any Tamil dictionary or lexicon.

That I have a special regard for Tamil is known by the many Tamil books I have written and published. Besides, I do not have any dispute with Tamil medical practitioners. The rifts due to their lack of unity, arguments and claims that their systems are different will only increase their present instability; we will be unable to enjoy the future benefits of being on an equal footing with western medicine. It is only to present this situation that I have included this topic in my address. After going through a few letters sent by the Secretary of this conference, I was impelled to give a detailed explanation of the situation on this occasion, so that my associate practitioners may be weaned away from their factional tendencies at least now and may function together as Āyurvedic physicians. The Āyurvedic physicians are ready to wholeheartedly welcome all the physicians in Tamilnadu, including those who identify themselves as practitioners of Siddha medical system.

Timelessness of Āyurveda

We have several grounds to believe that Āyurveda alone is the bedrock of all medical processes, that the western system of medicine undergoes frequent changes, that the omniscient sages have laid the foundation for Āyurveda in such a manner that the basic tenets remain immutable and hence it is the only medical system which has existed from ancient times and is permanent. Suśruta, one of the most prominent students of Dhanvantari himself, has declared Āyurveda to be an ancillary of the *Atharvan*◌ा*veda*. Agniveśa, who belonged to the Ātreya lineage, has said that 'Āyurveda is an ancillary to *R*◌ा*gveda* as well as *Atharvaveda*'. All the authors of the various Āyurvedic treatises and compilations have stated that Āyurveda is eternal and of divine origin. Vāgbhat◌ा has written that 'after recollecting the knowledge about Āyurveda, Brahmā initiated Daks◌ाprajāpati into it' (*As◌ा t◌ा āngahr◌ृ dayam* – *Sūtrasthānam* 1.3: '*brahmā smr◌ृ tvāyus◌ा o vedam◌ं prajāpatimajigrahat*'). Even the Buddhist, the opponents of the Vedic systems, referred to

medical science as ‘Āyurveda’; they never dared to either conceal or dispute the fact that the *Vedas* formed the basis of this science. Caraka states that ‘Brahmā brought to memory the sacred and eternal science of Āyurveda, which is based on the concept of *tridos*◌ा*a*’.

त्रिसूत्रं शाश्वतं पुण्यं बुबुधे यं पितामहः

(*Carakasam*◌ि*hitā – Sūtrasthānam* 1.24)

trisūtram◌ि *śāśvatam*◌ि *pun*◌ा*yam*◌ि *bubudhe yam*◌ि *pitāmahah*◌ा

The fact that we still identify ourselves by our *gotrams*, which are but the names of the sages – who lived many thousands of years ago, who had rid themselves of ego and possessiveness and who were engrossed in performing austerities – is proof that their pure blood continues to flow in our veins. If we pride ourselves as having been born in such a blemishless lineage and possessing mental and physical purity, if we believe in afterlife and have the ability to enjoy good health in the present without impeding the hereafter, then my request is that we should pledge to have faith in the divine *Veda* and its branch of Āyurveda and follow the methods mentioned therein and that, in our practice, we should eschew the differences of Āyurveda and Siddha and comport with love towards one another like brothers.

“One who desires a long life – which is an instrument to attain the three goals of altruism, prosperity and pleasure – should strongly support the teachings of Āyurveda”, says Vāgbhat◌ा a.

आयुः कामायमानेन धर्मार्थसुखसाधनम् | आयुर्वेदोपदेशेषु विधेयः परमादरः ||

(*As*◌ा*t*◌ा *āngahr*◌ृ*dayam – Sūtrasthānam* 1.2)

āyuh◌ा *kāmayamānena dharmārthasukhasādhanam* |
āyurvedopadeśes◌ा*u vidheyah*◌ा *paramādarah*◌ा ||

Our ancestors, *viz*. the sages, who possessed divine faculty, have surpassed people like us who are in the pursuit of wealth and pleasure; they had eliminated feelings of likes, dislikes and fears; they were eminently learned; their every uttering is an authority for us. It is these noble people who have taught us that the *Vedas* and Āyurveda are trustworthy testimonies. “The sages were wise... They were devoted to the path of virtue as a means to the highest knowledge... Scriptural testimony is that which is approved by those wise men and instituted for the wellbeing of mankind...; hence scriptural testimony is indeed based the *Vedas*”, “It is the gateway to Āyurveda”, says Agniveśa.

(ऋषयश्शिष्टाः, धर्मद्वारा अवहिताः, तादृश) शिष्टानुमतो लोकानुग्रहप्रवृत्तश्शास्त्रवादः .... आप्तागमः; (अतश्च) आप्तागमस्तावद्वेदः, (तद्द्वारा आयुर्वेदश्च) |[4]

(*Carakasaṁhitā – Sūtrasthānam* 11.27)

(*ṛṣayaśśiṣṭāh, dharmadvārā avahitāh, tādṛśa*) *śiṣṭānumato lokānugrahapravṛttaśśāstravādah ..... āptāgamah*; (*ataścā*) *āptāmagastāvadvedah, (taddvārā āyurvedaśca*) |

I request all of you not to hastily conclude that I am speaking about *Vedānta* that is irrelevant to today's worldly life. As it is extremely rare these days to find someone who has studied the books on Āyurveda texts from end to end, as we are steeped in the very prominent western culture and utterly lost our confidence and as the propagation of Āyurveda has fully stopped in South India for several years and so there is a danger of totally forgetting this knowledge, it is necessary to explain that Āyurveda is truly an outstanding, useful and eternal science.

Āyurveda – A Complete Medical System

Precepts for Physicians

I have mentioned earlier that, before writing texts on Āyurveda, our forebears, the sages, gathered in great numbers from time to time, thoroughly discussed the concepts and then recorded the findings and formulated the rules. While prescribing the eligibility criteria for a physician, it has been declared that one who (i) possesses an innate ability and intelligence, (ii) has learnt the scriptures under a teacher, (iii) has directly examined the diseases and watched the treatment methods of other physicians and (iv) maintains internal and external purity is considered a qualified physician (*Aṣṭāṅgahṛdayam – Sūtrasthānam* 1.28: '*dakṣah tīrthāttaśāstrārtho dṛṣṭakarmā śucirbhiṣak*'). Going through the guidelines advocated by Caraka regarding the conduct of doctors, there will be no room to consider today's culture as superior to that of the past.

मलायनेष्वभीक्ष्णं पादयोश्च वैमल्यमादध्यात्, त्रिः पक्षस्य केशश्मश्रुलोमनखान् संहारयेत्, नित्यमनुपहत-वासाः ....., साधुवेशः प्रसाधितकेशः ..... पूर्वाभिभाषी, सुमुखः, दुर्गेष्वभ्युपपत्ता ..... स्यात् |

(*Carakasaṁhitā – Sūtrasthānam* 8.18)

malāyaneṣvabhīkṣṇaṁ pādayośca vaimalyamādadhyāt, trih paksasya keśaśmaśruloma-nakhān saṁhārayet, nityamanupahatavāsāh ....., sādhuveśah prasādhitakeśah ..... pūrvābhibhāṣī, sumukhah, durgeṣvabhyupapattā ..... syāt |

[4] The words given in brackets are not found in the original Sanskrit text.

‘He should clean his excretory passages and feet very often; he should cut his hair, shave and cut his nails thrice every fortnight; he should always wear unsoiled clothes.....; he should be well dressed and have clean hair..... he should be the first to greet others, should be cheerful and should offer help to those in difficulty.’

On the basis of their qualities, Caraka has classified physicians into two – ‘*prān◌ाbhisara*’ (‘saviour of life’) and ‘*rogābhisara*’ (‘destroyer of life’) (*Carakasam◌्hitā – Sūtrasthānam* 29.4-9). One who has studied the science well, is well experienced, provides all kinds of help to the afflicted and respects and protects life is known as ‘*prān◌ाbhisara*’. Contrarily, one who has incomplete knowledge, aggressively pursues wealth and selfishly searches for patients (to ‘cure’ them for a fee) is called ‘*rogābhisara*’. Laying down the rules for a physician, Suśruta says, “He should have learnt this subject from a teacher, examined and verified the truths, acquired experience and the right based on his eligibility, obtained permission from the king and, with internal and external purity, should embark on his career”.

(आचार्यात्)[5] अधिगततन्त्रेण उपासिततन्त्रार्थेन दृष्टकर्मणा कृतयोग्येन ..... राजानुज्ञातेन ..... शुचिना ..... अकुहकेन ..... वैद्येन विशिखानुप्रवेष्टव्या |

(*Suśrutasam◌्hitā – Sūtrasthānam* 10.3)

(*ācāryāt*) *adhigatatantren◌ा upāsitatantrārthena dr◌ृs◌्t◌ाkarman◌ā kr◌ृtayogyena ..... rājānujñātena ..... śucinā ..... akuhakena ..... vaidyena viśikhānupraves◌्t◌ाvyā* |

Human Physiology

I have already said that Dr. Hornell and other modern western specialists had expressed surprise that the ancient science of Āyurveda contains very profound details about human physiology. Suśruta has given very lucid explanations about the human anatomy – starting from the skin and its layers such as *avabhāsinī*, *vedinī*, etc. to the seven kinds of *dhātus* (tissues) ending with *majjā* (marrow) and *śukra* (semen), parts of the abdomen such as *sūks◌्māntra* (small intestine) and *sthūlāntra* (large intestine), including *āmāśaya* (stomach), *pakvāśaya* (colon), *grahan◌ृī* (duodenum), etc., organs like *hr◌्dayam* (heart), *phuphphusa* (lungs), *klomā* (pancreas), *yakr◌ृt* (liver), *plīha* (spleen), *vr◌्kkā* (kidneys) etc., blood vessels like *dhamani* (arteries) and *sirā* (veins). Even the subject of *sūks◌्maśārīra* (concept of the ‘subtle body’) is not absent in the texts on Āyurveda.

Let us first take the topic of embryology. The structure of the uterus is described thus: “The vagina of women resembles the aperture of a conch. It has three *āvartas* (involuted sections).

[5] This word is not found in the original Sanskrit text.

The *garbhāśaya* (uterus) is located in the third section. The learned know the shape of the uterus as resembling the mouth of a Rohu carp (*Cyprinus Rohita*)".

शङ्खनाभ्याकृतिर्योनिस्त्र्यावर्ता सा प्रकीर्तिता | तस्यास्तृतीये त्वावर्ते गर्भशय्या प्रतिष्ठिता ||
यथा रोहितमत्स्यस्य मुखं भवति रूपतः | तत्संस्थानां तथारूपां गर्भशय्यां विदुर्बुधाः ||

(*Suśrutasaṁhitā – Śārīrasthānam* 5.43-44)

śankhanābhyākr॒tiryonistryāvartā sā prakīrtitā |
tasyāstr॒tīye tvāvarte garbhaśayyā pratis॓t॓hitā ||
yathā rohitamatsyasya mukham॑ bhavati rūpatah॓ |
tatsam॑sthānām॑ tathārūpām॑ garbhaśayyām॑ vidurbudhāh॓ ||

मासेनोपचितं काले धमनीभ्यां तदार्तवम् | ईषत्कृष्णं विगन्धं च वायुर्योनिमुखं नयेत् ||

(*Suśrutasaṁhitā – Śārīrasthānam* 3.10)

māsenopacitam॑ kāle dhamanībhyām॑ tadārtavam |
īs॓atkr॒s॓n॓am॑ vigandham॑ ca vāyuryonimukham॑ nayet ||

"The menstrual fluid collected over the past one month is carried by *vāyu* through the fallopian tubes to the mouth of the uterus, whence it flows out, odourless and dark in colour."

गृहीतगर्भाणामार्तववहस्रोतसां वर्त्मान्यवरुध्यन्ते गर्भेण (पाटवञ्च स्रोतसां प्रायेण मन्दीभवति)[6] | तस्माद्गृहीतगर्भाणामार्तवं न दृश्यते |

(*Suśrutasaṁhitā – Śārīrasthānam* 4.24)

gr॒hītagarbhān॓āmārtavavahasrotasām॑ vartmānyavarudhyante garbhen॒a (pāt॓avañca srotasām॑ prāyen॓a mandībhavti) | tasmādgr॒hītagarbhān॓āmārtavam॑ na dr॒śyate |

"The openings of the uterine channels of a pregnant woman are blocked by the foetus, (the intensity of the flow usually slows down). Hence menses are not seen in a pregnant woman."

मातुस्तु खलु रसवहायां नाड्यां (अपराभिधायां)[7] गर्भनाभिनाडी प्रतिबद्धा सास्य मातुराहार-रसवीर्यमभिवहति |

(*Suśrutasaṁhitā – Śārīrasthānam* 3.31)

mātustu khalu rasavahāyām॑ nād॓yām॑ (aparābhidhyāyām॑) garbhanābhinād॓ī pratibaddhā sāsya māturāhārarasavīryamabhivahati |

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[6] The words in brackets are not found in the original Sanskrit text.

[7] The word in brackets is not found in the original Sanskrit text.

“The umbilical cord of the foetus is attached to the womb of its mother by arteries, through which the essence of the food assimilated by her enters the foetus and causes it to develop.”

Besides the above information, many other points have been very clearly explained in the *Suśruta-samhitā*. More than all these, it is the concept of the *śārīra* that is pre-eminent in Āyurveda.

Blood

The following verses show how thoroughly our sages had examined the element blood.

रञ्जितास्तेजसा त्वापः शरीरस्थेन देहिनाम् | अव्यापन्नः प्रसन्नेन रक्तमित्यभिधीयते ||
(*Suśrutasamhitā – Sūtrasthānam* 14.5)

rañjitāstejasā tvāpaḥ śarīrasthena dehinām |
avyāpannaḥ prasannena raktamityabhidhīyate ||

“The lymphatic fluid which acquires red colour and heat due to the element of fire in a normal, healthy body is known as ‘blood’.”

..... जन्तवोऽण्वः | अपादा वृत्तताम्राश्च सौक्ष्म्यात् केचिददर्शनाः ||
(*Asthyangahrdayam – Nidānasthānam* 14.51)

..... jantavo’ṇvaḥ | apādā vṛttatāmrāśca saukṣmyāt kecidadarśanāḥ ||

“Some of the blood-cells are without limbs; they are circular and red in colour. As they are extremely tiny, they will not be visible to the eye.”

The reason for our sages acquiring and documenting this highly technical knowledge – either due to their superior insight or by some other means – cannot be attributed to Leeuwenhoek (Antonie van Leeuwenhoek) or Malpighi (Marcello Malpighi).

Besides, in the *Śonitavarṇanīyādhyāya* (chapter 14 of the *Sūtrasthānam*), Suśruta says, “The blood has the heart as its seat; it flows to all parts of the body through the blood vessels that branch out and go upwards, downwards and lateral, thus constantly nourishing and protecting the body and maintaining life; the manner in which blood flows can be understood from its continuous coursing throughout the body”.

तस्य ..... हृदयं स्थानं; स हृदयात् ..... धमनीरनुप्रविश्य ऊर्ध्वगा ..... अधोगा ..... तिर्यग्गाः (च धमन्यः)[8] कृत्स्नं देहमहरहस्तर्पयति ..... जीवयति च.....| तस्य शरीरमनुधावतो ..... गतिरुपलक्षयितव्या ..... |

(*Suśrutasaṁhitā – Sūtrasthānam* 14.3)

tasya ..... hṛdayaṁ sthānaṁ; sa hṛdayāt ..... dhamanīranupraviśya ūrdhvagā ..... adhogā ..... tiryaggā (*ca dhamanyaḥ*) *kṛtsnaṁ deham aharahastarpayati ..... jīvayati ca; tasya śarīram anudhāvato ..... gatirupalakṣayitavyā ..... |*

Thus it can be seen that the sages reckoned blood to be continually flowing throughout the body; nowhere did they say that it stagnated in one place like water in a pond. So one can imagine how absurd it is to claim that, until William Harvey discovered in 1616 B.C., nobody knew that blood flowed in the body through the vessels.

The Three Humours

Physiology is the science that deals with the natural processes and functions of living beings. In Āyurveda, this has been defined as a study of the elements of *tridoṣas* (three humours of *vāta*, *pitta* and *kapha*) and the aspects of the Sānkhya philosophy. This concept forms the basis to understand the two states of all creatures – their bodily equilibrium when healthy and imbalance when diseased. This is also the prominent aspect that demarcates Āyurveda and western medical system, thus highlighting the basic difference between them. The theory of *sūkṣmaśārīra* is related to the philosophical systems of Sānkhya and Yoga. So, even though the bodies of all creatures are made of the five elements, when nature tries to make these elements visible, the principle of *bhūtādi* (the egoity principle) and the *tanmātras* (the subtle objects of the sense-organs) – neither of which can be perceived even by the present-day microscope – should also transform into gross state (so that they too can be comprehended). The subtle element of *vāyu*, by associating with the quality of *rajas*, causes itself to transform into a gross state. It is also instrumental in making this body – which is but a result of such a transformation – to grow, helping the limbs and other parts to fulfil the bodily functions and bestowing *caitanya* (consciousness). This *vāta*, which is a natural product of the *tanmātras* (subtle elements) of *ākāśa* and *vāyu* during the process of creation by Bhagavān, wields complete authority in our body. It is the locus of sensation, enthusiasm, awareness, etc. While the two humours of *pitta* (bile) and *kapha* (phlegm) have their own qualities and purposes, it is *vāyu* which controls (the functioning of) these two as well as the entire body. Noticing its importance, Suśruta has therefore emphasised its unusual qualities thus: "Self-created and identified with (the divine energy of) God, it is known as '*vāyu*'. Due to its independence, eternality and all-pervasiveness, it is regarded as the universal soul and worshipped by the entire world. It is the reason for the existence, origin and disintegration of all created beings. Though invisible by itself, its work is revealed. It is dry, cold, light and sharp".

[8] The words in brackets are not found in the original Sanskrit text.

स्वयम्भूरेष भगवान् वायुरित्यभिशब्दितः ||
स्वातन्त्र्यान्नित्यभावाच्च सर्वगत्वात् तथैव च | सर्वेषामेव सर्वात्मा सर्वलोकनमस्कृतः ||
स्थित्युत्पत्तिविनाशेषु भूतानामेष कारणम् | अव्यक्तो व्यक्तकर्मा च रक्षः शीतो लघुः खरः ||
(*Suśrutasaṁhitā – Nidānasthānam* 1.5-7)

svayambhūreṣa bhagavān vāyurityabhiśabditaḥ ||
svātantryānnityabhāvācca sarvagatvāt tathaiva ca |
sarveṣāmeva sarvātmā sarvalokanamaskṛtaḥ ||
sthityutpattivināśeṣu bhūtānāmeṣa kāraṇam |
avyakto vyaktakarmā ca rukṣaḥ śīto laghuḥ kharaḥ ||

The two humours of *pitta* and *kapha* originate respectively from the elements of *tejas* (fire) and *ap* (water). These two also perform some functions in the body with their natural ability. It is the *vāyu* which administers the components of flesh, fat, marrow and other similar gross entities in their respective tasks. Just as steam or electricity causes an appliance to function, the *vāyu* impels all the actions in our body. The words '*vāta*' and '*vāyu*' originate from the verbal root '*vā – gati-gandhayoḥ*'. Going by the meaning '*gati*' ('going'), *vāyu* effects all actions – both involuntary and voluntary – in the body; based on the meaning '*gandha*' ('blowing'), it causes all responses in the body and mind. This is one of the many aspects related to physiology in Āyurveda.

Maintenance of Good Health

Āyurveda holds that maintaining good health is foremost and more important than treating a person after the onset of a disease. This approach is known as '*Svasthrvṛttam*'. Caraka states, "The purpose of this system is to maintain the health of a healthy individual" (*Carakasaṁhitā – Sūtrasthānam* 30.26: '*svasthasya svāsthyarakṣaṇam*').

Underscoring this stand, Suśruta says, "The primary aim of medical treatment is to maintain the normal equilibrium of the healthy state".

सूत्रस्थाने समुद्दिष्टः स्वस्थो भवति यादृशः | तस्य यद्रक्षणं तद्धि चिकित्सायाः प्रयोजनम् ||
(*Suśrutasaṁhitā – Uttaratantram* 64.3)

sūtrasthāne samuddiṣṭaḥ svastho bhavati yādṛśaḥ |
tasya yadrakṣaṇaṁ taddhi cikitsāyāḥ prayojanam ||

All the methods employed in carefully managing good health are given in the sections on *dinacaryā* (daily regimen) and *ṛtucaryā* (seasonal regimen)[9]. Right from the injunction that

[9] These sections form the 2nd and 3rd chapters of the *Aṣṭāṅgahṛdayam – Sūtrasthānam*.

‘a healthy individual who wishes to guard his longevity and remain disease-free should get up at the *Brahmamuhūrta*’, to one’s personal hygiene, care of teeth, changes in food and activities corresponding to the seasons and other essential aspects – all these have been extensively covered in Āyurveda.

ब्राह्मे मुहूर्ते उत्तिष्ठेत् स्वस्थो रक्षार्थमायुषः

(*As॒t॒āngahr॑dayam – Sūtrasthānam* 2.1)

brāhme muhūrta uttis॒t॒het svastho raks॒ārthamāyus॒ah॒

The subject of *Svasthavr॑ttam* (maintenance of good health), which is discussed in the *As॒t॒āngahr॑dayam*, specially mentioned in the *Carakasam।hitā* and specifically dealt with in the *Svasthavr॑ttādyadhyāya* of the *Suśrutasam।hitā*, makes for interesting reading. Caraka has classified bodily strength into three – ‘*sahajabalam*’ (inherent stamina), ‘*kālajabalam* (strength due to change of seasons and age) and ‘*yuktikr॑tabalam*’ (stamina that is augmented by exercises, etc.) – and has explained ways of preserving it.

That physical exercise is the best way to preserve good health and to increase stamina has been mentioned in all the books on Āyurveda. Vāgbhat॒a says, “Exercise should be performed daily as it delays aging, relieves fatigue and mitigates *vāta*”.

(व्यायामम्)[10] आचरेन्नित्यं स जराश्रमवातहा |

(*As॒t॒āngahr॑dayam – Sūtrasthānam* 2.8)

vyāyāmam ācarennityam। sa jarāśramavātahā

The results of exercise are specifically given thus: “Subjecting the body to labour to the point of exertion is called ‘exercise’. After completing the exercises, the body should be massaged. Exercising thus will increase physical stamina; the body will have a kind of radiance. All the limbs will become sturdy. The body will acquire the power to withstand heat, cold and fatigue. There is no better treatment for obesity. He (who exercises regularly) will easily be able to digest even heavy food. Even an ugly person will appear handsome (if he performs exercises daily)” (Refer *As॒t॒āngahr॑dayam – Sūtrasthānam* 2.10-12).

Aetiology

Just like the concept of *tridos॒a*, the diagnostic methods such as *nidānapañcaka*, etc. are also unique features of Āyurveda and cannot be found in any other system of medicine. Since any disturbance in the equilibrium of the three *dhātus* (humours) of *vāta*, *pitta* and *kapha* – which maintain the health of the body (they support the body and are therefore called

[10] The original Sanskrit text has the word ‘*abhyangam*’ instead of ‘*vyāyāmam*’.

'*dhātus*'[11]) – harms the body, the impaired state of the *dhātus* is called '*dos॰a*'[12]. The symptoms caused by disturbances resulting from either an increase or decrease in the *dos॰a* have been explained separately. The physician should keenly observe the symptoms in the patient's body and ascertain the cause of the disease – whether it is '*sannikr॰s॰t॰anidānam*' (direct), *viprakr॰s॰t॰anidānam* (indirect or inherited), '*asātmiyendriyārthajanyanidānam*' (subjecting the body to unusual situations), etc. – or other reasons (such as *parin॰āmajanidānam*, *prajñāparādhajanidānam*, *vyābhicārikanidānam*, *utpādakanidānam*, *vyañjakanidānam*).

Here, I will give an example for just one. In the ailment of '*āmavāta*' (rheumatoid arthritis) (in Tamil, it is called '*kīl vāyu*'), since it is the '*āma*' ('toxins') which causes this condition, it is the '*utpādakahetu*' ('effective cause'); since the external symptoms of this ailment are due to the impairment of *vāyu* (wind) and *kapha* (phlegm), these two are known as '*vyañjakahetu*' ('indicative causes') for this condition. Similarly, the concepts of (i) '*rūpam*' – symptoms of the (main) disease, (ii) '*pragrūpam*' – initial indications which appear prior to the symptoms of the main disease, (iii) '*upacaya*' – the technique that helps in diagnosis in cases where the symptoms pertain to two or three diseases, by prescribing different medicines and observing the results, thereby correctly identifying the disease and (iv) '*samprāpti*' – pathogenesis or the origin and development of the disease, right from its onset in the body, its internal and external symptoms and its various stages of progression – have all been explained, along with their several categories, in the Āyurvedic texts, revealing the immense insight our physicians had thousands of years ago in methods of diagnosis, which is a matter of great pride for us.

Eightfold Examination

In diagnosing a disease using the process of *nidānapañcaka*, mentioned earlier, three important methods are advocated – inspection, palpation and interrogation.

दर्शनस्पर्शनप्रश्नैः परीक्षेताथ रोगिणम्

(*As॰t॰āngahr॰dayam – Sūtrasthānam* 1.22)

darśanasparśanapraśnaih॰ parīks॰etātha rogin॰am

In matters of discerning the malady, there is a marked difference in the observations of a patient by an ordinary man and that by an experienced doctor. The learned doctor, with his keen intellect, can accurately perceive the real difference between a healthy man and a sick

[11] The verbal root of the word '*dhātu*' is '*dhā*' which means 'to support' or 'to nourish' (*dhā dhāran॰a-pos॰an॰ayoh॰*). Hence the word '*dhātu*' means 'that which supports or nourishes'.

[12] The word '*dos॰a*' comes from the root '*dus॰*', which means 'to injure' or 'to spoil'.

man through their body colours, postures while seated or supine, their faces, eyes and breathing. This is the objective of visual examination. The difference between the natural sounds produced by healthy organs and the contrasting sounds of impaired organs can be noted either by palpation or by auscultation. The fact that this method has been mentioned (in our texts) shows that the basic concept of using sound to identify good health or otherwise is not new to us.

Many people are amazed by the use of a stethoscope for auscultation, not knowing that it is only an enhancement of the procedure already available to us. The methods of palpation and interrogation are also useful to physicians in several ways in diagnosing a disease. The mode of '*nād͡ īparīks͡ ā*' (feeling the pulse of a patient) is a unique method developed by sages who were physicians and also practitioners of *yoga*. The works '*Nād͡ īvijñānam*' and '*Nād͡ īnirn͡ aya*' in Sanskrit and the '*Tirumūlar Nād͡ i*' in Tamil are very useful for this method of examination. This technique (of diagnosis by feeling a person's pulse) was earlier an independent method, which was added to the medical texts by Śārngadhara, the physician of the 13th century.

A good knowledge of this procedure can be gained only by studying the related texts and acquiring specialised experience; it is impossible to acquire expertise through a little practice. Merely checking the pulse-rate per minute and observing the variations in the count, etc. will not indicate full knowledge in pulse examination. When a skilled musician plays the *vīn͡ ā* (South Indian chordophone instrument), he will discern and enjoy every subtle nuance of the musical notes produced by the vibrating strings; if a person who has passed a test in telegraphy attempts to pluck at the strings of the instrument in the way he has learnt, but is unable to hear any tapping sound that he had studied about (in telegraphy), is it proper to criticise either the musician's knowledge or the melodiously delicate sounds of the *vīn͡ ā*?

Besides these (the *nād͡ īparīks͡ ā*), our learned ancestors have also employed and documented the methods of examining the tongue, the eye, the stool and the urine and other basic techniques of diagnosing a disease. Treatment should not be begun at whim, but should be preceded by a careful examination and diagnosis of the ailment and selection of the appropriate medicine.

रोगमादौ परीक्षेत ततोऽनन्तरमौषधम् | ततः कर्म भिषक् (कुर्याद्व्याधिरेवं प्रशाम्यति)[13] ||
(*Carakasam͡ hitā – Sūtrasthānam* 20.20)

rogamādau parīks͡ eta tato'nantaramaus͡ adham |
tatah͡ karma bhis͡ ak kuryādvyādhirevam͡ praśāmyati ||

[13] The words in brackets are not found in the original Sanskrit text.

Early physicians have maintained that only the treatment which does not transgress the above injunction will be effective in completely eradicating the disease. Realising that examination and diagnosis of a disease prior to treatment is the duty of the physician and that diagnosis is of paramount importance, our physicians have mentioned this topic separately, besides composing independent texts on the subject, such as '*Mādhavanidānam*'.

Course of Treatment

The term '*āturavr्ttam*' is the general name for all methods of treating a disease. The system of daily activities practised by healthy people is referred to as '*svasthavr्ttam*'; this has already been explained earlier. The establishments which are now known as 'hospitals', were earlier called '*āturālaya*' by Āyurvedic practitioners. The matter did not stop with the mere injunction that treatment should commence after examining the diseases. The duty of the physician is to meticulously and thoroughly examine the element or elements that are afflicted, the body-tissues that are impaired by the elements, the part that is mainly affected in the body, the physical stamina of the patient, the duration of treatment, the patient's digestive condition, his constitution, age, mental state, food-habits and other numerous related aspects and finally locate the defective elements and then decide on the most appropriate treatment.

दूष्यं देशं बलं कालमनलं प्रकृति वयः | सत्त्वं सात्म्यं तथाहारमवस्थाश्च पृथग्विधाः ||
सूक्ष्मसूक्ष्माः समीक्ष्यैषां दोषौषधिनिरूपणे | यो वर्तते..... ||

(*Ast ängahr्dayam – Sūtrasthānam* 12.67-68)

dūsyam deśam balam kālamanalam prakr्ti vayah |
sattvam sātmyam tathāhāramavasthāśca pr्thagvidhāh ||
sūksmasūksmāh samīksyais ām dos aus adhinirūpan e |
yo vartate..... ||

Thus our medical system has laid down the rules for a meticulous and careful commencement of treatment. Aware that even a short delay in treatment can create risks and problems, they (our early physicians) have also said, "No time should be lost (in commencing treatment)" (*Suśrutasamhitā – Sūtrasthānam* 35.21: '*kriyākālam na hāpayet*').

That the systems of treatment are also based on unique techniques is generally not known to many. The process of curing a disease by expelling toxins from the body is '*śodhanam*' (detoxification), while treating an ailment by restoring the affected elements to the state of equilibrium is called '*śamanam*' (pacification); it can be said that several other treatment methods are built upon the foundation of these two techniques. The procedures of (1) *virechanam* (purgation), *vamanam* (emesis), *nasyam* (administration of drugs through the nasal cavity), *nirūhavasti* (administration of herbal decoction as enema), *anuvāsanavasti*

(usage of oil as enema) and *raktamoks◌̃an◌̃am* (blood-letting) are purificatory techniques based on the first process, *viz*. *śodhanam*, while the therapies of (2) *pācanam* (administration of carminative), *dīpanam* (administration of digestive), *langhanam* (fasting), *snehanam* (oleation), *vyāyāma* (exercise), *mardanam* (massage) and *paris◌̃eka* (bath) have the second process, *i.e*. *śamanam*, as their basis.

Generally, the prescribed mode of treatment is that, in situations where the disease cannot be eradicated without removing the toxins or impurities in the humours or the respiratory and urinary or the digestive organs, one of the techniques listed under *śodhanam* is used; (on the other hand) if the impurity, etc. are insignificant and if the patient cannot withstand the methods of *śodhanam*, then one of the therapies given under *śamanam* is used in treatment. Even though *śodhanam* and *śamanam* are the two main therapeutic processes for eradicating a disease, the system of treatment used to strengthen the body or to gain weight and to improve the health of the sense-organs is broadly known as '*br◌̽m◌̐han◌̃am*' (nourishment). Under this are *rasāyanam* (rejuvenation) and *vājīkaran◌̃am* (treatment of sexual diseases and promotion of sexual health).

We need not think that only the new system of medicine has different methods of treatment. Since *vamanam*, *virecanam*, *vasti*, *rakta-moks◌̃an◌̃am*, etc. are methods of *śodhanam* intended to purify the internal organs, Caraka refers to them as '*antah◌̃parimārjanacikitsā*', while the *śodhanam* techniques of *abhyanga* (oil-massage), *svedanam* (steam-therapy), *pradeha* (fomentation), *secanam* (irrigation), *unmardanam* (upward massage), etc. are used to cleanse the external organs, he classifies them as '*bahih◌̃mārjanacikitsā*'. Other Āyurveda preceptors group and study these two sections as a single topic, named '*kāyacikitsā*'. There is another method of classification, wherein the treatments using medicines and exercise come under *kāyacikitsā*, whereas those involving instruments, alkalis, leeches and apparatuses are labelled as '*śastracikitsā*' (or '*śalyacikitsā*').

Among the notable (Āyurveda) works today, the *Carakasam◌̐hitā* is said to be the main book for *kāyacikitsā*; the *Suśrutasam◌̐hitā* is cited as a prominent text for *śastracikitsā*. Both these processes have been extensively explained in the well-written and invaluable book, the '*As◌̃t◌̃āngahr◌̽dayam*' of Ācārya Vāgbhat◌̃a. This book has another special feature. Though the eight types of treatment – *kāyacikitsā* (general medicine), *bālacikitsā* (paediatrics), *grahacikitsā* (disorders of the mind and those caused by the planets), *ūrdhvāngacikitsā* (treatment of diseases in organs above the shoulders, *viz*. nose, eyes, ears, face, throat, etc.), *śalyacikitsā* (surgery), *dams◌̃t◌̃racikitsā* (toxicology), *jarācikitsā* (geriatrics) and *vr◌̽s◌̃acikitsā* (aphrodisiac therapy) – have been mentioned by Caraka and Suśruta, it is Vāgbhat◌̃a who has serialised and fully explained them, thus helping future generations like us.

The discerning individuals who take the effort to study our texts that contain numerous methods of treatment mentioned earlier, together with their subdivisions such as *vātātapika-cikitsā* (outdoor rejuvenation therapy) and *kut॰īpraveśa-cikitsā* (indoor rejuvenation therapy that requires isolation), *jentāka* (dry hot bath, to induce perspiration) and *śirovasti* (pouring and retaining of medicated oils on the head), *sirāvyadha* (venesection), etc. will come to the conclusion that Āyurveda is the source of all treatment methods required to cure diseases.

Cold and Hot Remedies

There is a wrong notion among some that only the western physicians practised the method of applying ice to reduce fever. That this technique is very important in treating conditions like *abhinyāsa-jvara* (fever), *āntrika-sannipāta-jvara* (diseases of the intestinal tract), *sannyāsa* (coma) has been described in Āyurvedic books thousand years ago. Ācārya Vāgbhat॰a, who lived before the 6th century, has advocated that 'to reduce acute fevers and to mitigate severe vomiting, very cold water should be gently poured continuously on the head'.

जेतुं ज्वरान् प्रविषमानिह वान्तिशान्त्यै | मौलौ सुशीतलजलस्य ददीत धाराम् ||
(*Rasaratnasamuccaya* 12.32)

jetum‹ jvarān pravis॰amāniha vāntiśāntyai | maulau suśītalajalasya dadīta dhārām ||

Let the audience themselves surmise from this as to who learnt these techniques from whom. For the distinguished preceptors who composed texts on Āyurveda, envisioning such methods of treatment was but effortless as, while recording the laws and bylaws required for treatment, they would have naturally thought of many such concepts.

Thus, to prescribe the remedy of pouring very cold water on the head in case of fevers, a general rule has been laid down that 'diseases caused due to excess heat should be allayed by cold remedy; ailments caused by coldness should be treated through heat remedy. Physicians doing so will succeed in their effort'.

शीतेनोष्णकृतान् रोगाञ्छमयन्ति भिषग्विदः | ये तु शीतकृता रोगास्तेषामुष्णं भिषग्जितम् ||
(*Carakasam‹hitā – Vimānasthānam* 3.41)

śītenos॰n॰akr॰tān rogāñchamayanti bhis॰agvidah॰ |
ye tu śītakr॰tā rogāstes॰āmus॰n॰am bhis॰agjitam ||

While describing many other cold remedies, Caraka has specifically mentioned the use of ice-water ('*candanapankaṁ śuśītamālimpet; śirasi himodakaṁ vā muhurmuhussiñcet*'[14]).

Sūcikābharaṇam

I will give another example to show the complete knowledge of the ancient and ingenious medical preceptors in intuiting several techniques and implementing them during the course of treatment. Vāgbhaṭa has said, "In cases of utter collapse, a small quantity of the drug *Mṛtasañjīvini*, prepared according to procedure, should be mixed well with water and the solution administered to the patient through an incision on his palate, done with a very sharp needle, thus causing the liquid to mix with his blood; this method will revive the patient in a life-threatening situation. This practice of piercing with a needle is called '*sūcikābharaṇam*'".

मृतसञ्जीवनाख्योऽयं सूचिकाभरणो रसः ॥
सन्निपातेन तीव्रेण मुमूर्षोर्भूगतस्य च | तालुनि प्रच्छयित्वाथ रसमेनं विनिक्षिपेत् ॥
सूच्यातिसूक्ष्मया (तोययुक्तया रक्तमिश्रितः)[15] |

Rasaratnasamuccaya 12.74-76

mṛtasañjīvanākhyo'yaṁ sūcikābharaṇo rasaḥ ||
sannipātena tīvreṇa mumūrṣorbhūgatasya ca |
tāluni pracchayitvātha rasamenaṁ viniksipet ||
sūcyātisūksmayā (*toyayuktayā raktamiśritaḥ*) |.

The medicine or the method of puncturing the body with the tip of a needle and allowing the drug to enter the bloodstream at that spot is called '*sūcikābharaṇam*' or '*sūcīmukhaprayoga*' in Āyurveda. It is also said in the *Rasaratnasamuccaya* that 'making a small cut at the '*brahmadvāra*' or the central point of the skull with the point of a needle and injecting a rosary pea-sized amount of the medicine called *sūcīmukham* into the bloodstream will at once cure diseases such as coma, tinnitus, tetanus and epilepsy'.

..... रसवरं सूचीमुखं नामतः ॥ ब्रह्मद्वारविकीर्णलोहितलवं गुञ्जैकमातरं ददेत्

(*Rasaratnasamuccaya* 12.92-93)

... rasavaraṁ sūcīmukhaṁ nāmataḥ ||
brahmadvāravikīrṇa alohitalavaṁ guñjaikamātraṁ dadet

[14] Source is not traceable for this line.

[15] The words in brackets are not found in the original Sanskrit text.

Hence, despite our ancestors discovering hundreds of years ago that, besides being ingested and given as enema through the anal opening, if medicines are injected into the bloodstream, they would reach the organs and easily effect a cure, our doctors failed to read them carefully and use the techniques. Even though a few minor techniques included in the (Āyurvedic) medical system appear unique to those exposed to western medicine, our earlier preceptors had prepared the ground for these techniques, sowed the seeds and produced seedlings, as it were, while the opportunities and facilities to easily nurture them and fully enjoy their benefits were conducive to others.

Surgery

That we have neglected our dexterity in surgery for a very long time is a mistake committed by our intermediate generation. It has been stated in the Usman Committee report itself that the Āyurvedic system is complete in all respects and that, for some time, assistance has to be sought from the western medical system in surgery alone. However, our early preceptors (in Āyurveda) have fully known all the necessary aspects of surgery, gained experience through practice and also written books on them. The surgical procedures conducted below the neck are known as '*sāmānya-śalyacikitsā*', while those done above the neck are called '*śālākya-cikitsā*', thus classifying them into '*sāmānya*' (normal) and '*viśes*◌̃*a*' (specialised) surgery. If we carefully read the numerous topics on surgery and the related guidelines that were written by our Āyurvedic teachers, we will have to deeply regret not having studied them.

The devices used in surgery are generally divided into '*yantras*' (machines or apparatuses) and '*śastra*' (instruments) by Suśruta and Vāgbhat◌̃a, who have given the further groupings of each *yantra* and *śastra* and also precisely described their characteristics and uses. Leave alone the surgery required to open an ordinary abscess, the experienced description by Suśruta and others of surgical procedures for *chidrodaram*, *parisrāvyudaram* (intestinal perforation), *jalodaram* (ascites or accumulation of fluid in the peritoneal cavity), etc., which involve cutting open the abdomen and using instruments, make for an interesting reading.

During childbirth, the four wrong positions of a foetus are: (i) *kīla* (the foetus presents itself with its back to the vaginal opening, with its arms, head and feet turned upwards and inside the womb), (ii) *pratikhura* (the hands, feet and head of the foetus come out, but the body is impacted at the entrance), (iii) *bījaka* (when the head and one arm of the foetus come out, but the passage of the rest of the body is obstructed) and (iv) *parigha* (the foetus gets obstructed horizontally across the birth canal like an iron bar) [16] – collectively known as '*mūd*◌̃*hagarbha*' (obstructed foetus) or '*vikr*◌̃*taprasava*' (dystocia). In such situations, the foetus should be repositioned correctly and removed without harming the mother.

[16] These classifications are according to the *Suśrutasam*◌̃*hitā – Nidānasthānam* (9.4)

"In the irregular position called '*pratiloma*' (when the foetus is dead and in reverse), the (physician's) hand, greased with a mixture of the gum from the silk-cotton tree and ghee, should be inserted (into the mother's vagina) to turn the foetus in the right direction and firmly pull it out" (*As◌̃t◌̃āngahr◌̃dayam – Śārīrasthānam* 2.28: '*ānulomya samākars◌̃edyonim◌̃ pratyārjavāgatam*' and *Ibid*. 2.27: '*sājyaśālmalipicchayā hastena*').

In the abnormal position called *vis◌̃kambha* (where the hands, feet or head of the dead foetus is impacted during descent at the mother's genital tract or when one leg of the foetus comes out through the mother's vagina and the other through her rectum), the foetus should be removed with the help of instruments such as *anguliśastram* (finger-knife), *man◌̃d◌̃alāgram* (circular knife)[17], the latter being used to sever the obstructing parts (of the foetus) and bring it out. All these techniques should be studied by the modern doctors. Some of the remedies mentioned in the *As◌̃t◌̃āngahr◌̃dayam* for this foetal malpresentation are: "If the foetus has a very big head which prevents its passage from the womb, then the skull should be broken and the entire body of the foetus should be fully extracted; a very skilled surgeon is required to perform this operation; after breaking the skull thus, the foetus should be pulled out using an instrument called '*garbhaśanku*' (foetal probe), which is hooked to the armpit or chest or cheek or chin or to any other convenient part; the foetus whose (obstructing) skull is not broken should be pulled out with the same instrument, which is fastened to either the cheek or the eye-sockets; if the shoulder of the foetus impedes its passage, its arm should be cut; if the abdomen of the foetus is bloated with gas and causes obstruction, its abdomen should be cut open and the entrails pulled out; if the hip of the foetus causes hindrance, the hipbone should be broken; whichever part prevents its passage should be cut into small pieces and the foetus removed completely without causing any harm to the mother and to protect her".[18]

पूर्वं शिरःकपालानि दारयित्वा विशोधयेत् ||
कक्षोरस्तालुचिबुकप्रदेशेऽन्यतमे(ऽपि वा)[19] | समालम्ब्य दृढं कर्षेत्कुशलो गर्भशङ्कुना ||
अभिन्नशिरसं त्वक्षिकूटयोर्गण्डयोरापि | बाहुं छित्वांससक्तस्य वाताध्मातोदरस्य च ||
विदार्य कोष्ठमन्त्राणि बहिर्वा सन्निरस्य च | कटीसक्तस्य तद्वच्च तत्कपालानि दारयेत् ||

यद्यद्वायुवशादङ्गं सज्जेद्गर्भस्य खण्डशः | तत्तच्छित्वाहरेत् सम्यग्रक्षेन्नारीं च यत्नतः ||

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[17] Please refer *As◌̃t◌̃āngahr◌̃dayam – Śārīrasthānam* 2.29-30.

[18] This paragraph and the previous one have been transposed for continuity. Both of them deal with situations where the foetus is dead. Please refer *As◌̃t◌̃āngahr◌̃dayam – Śārīrasthānam* 2.23. As these two abnormal presentations (*pratiloma* and *vis◌̃kambha*) are mentioned after this verse, they are taken to describe the methods of removing a dead foetus.

[19] The original Sanskrit text has the word '*tatah◌̃*' instead of '*api vā*'.

(*As॒t॑āngahr॓dayam – Śārīrasthānam* 2.31-35)

pūrvam॑ śirah॒ kapālāni dārayitvā viśodhayet |
kaks॒orastālucibukapradeśe'nyatame'pi vā |
samālambya dr॓dham॑ kars॒etkuśalo garbhaśankunā ||
abhinnaśirasam॑ tvaks॒ikūt॑ ayorgan॒d॒ayorapi |
bāhum॑ chitvāmsasaktasya vātādhmātodarasya ca ||
vidārya kos॒t॑amantrān॒i bahirvā sannirasya ca |
kat॑īsaktasya tadvacca tatkapālāni dārayet ||
yadyadvāyuvaśādangam॑ sajjedgarbhasya khan॒d॒aśah॒ |
tattacchitvāharet samyagraks॒ennārīm॑ ca yatnatah॒ ||

Fracture

Among the many branches of surgery, when we study the section dealing with fractures and their treatment, we will feel that it is even more fascinating.

1. "Bones can sustain dislocation or fracture either at the joints or elsewhere. The causes for these are fall or assault. In dislocation, extending and flexing (the limb) becomes difficult. If the bone is broken, excessive swelling, intense pain, inability to bear even the slightest movement and a grating sound during palpation are the symptoms in brief. Fractures are of many kinds. They have been severally classified into *vidārita* (split), *ks॒un॒n॒a* (shattered), *bhinna* (broken) and *viślis॒t॑a* (dislocated).

पातघातादिभिर्द्वेधा भङ्गोऽस्थ्नां सन्ध्यसन्धितः | प्रसारणाकुञ्चनयोरशक्तितः सन्धिमुक्तका ||
इतरस्मिन् भृशं शोफः सर्वावस्थास्वतिव्यथा | अशक्तिश्चेष्टितेऽल्पेऽपि पीड्यमाने सशब्दता ||
समासादिति भङ्गस्य लक्षणं बहुधा (च)[20] तत् |
(*As॒t॑āngahr॓dayam – Uttarasthānam* 27.1-3)

pātaghātādibhirdvedhā bhango'sthnām॑ sandhyasandhitah॒ |
prasāran॒ākuñcanayoraśaktih॒ sandhimuktakā ||
itarasmin bhr॓śam॑ śophah॒ sarvāvasthāsvativyathā |
aśaktiśces॒t॑ite'lpe'pi pīd॒yamāne saśabdatā ||
samāsāditi bhangasya laks॒an॒am॑ bahudhā ca tat |

2. While describing fractures that are difficult to treat (they are called '*kas॒t॑asādhya*'), Vāgbhat॑a says, "When the bone is broken into many small pieces or when the bone

[20] The original Sanskrit text has the word '*tu*' instead of '*ca*'.

makes a sound while being examined or when a fragment of a bone has pierced another bone and embedded inside it or when, due to trauma, a major part of the bone is shattered and only a small segment is left or when the fractured bone, while being removed, causes pain as though wounded and gets lodged in the marrow – these are complicated conditions".

प्राज्याणुदारि यत्त्वस्थि स्पर्शे शब्दं करोति यत् ||
यत्रास्थिलेशः प्रविशेन्मध्यमस्थ्नो विदारितः | भग्नं यच्चाभिघातेन किञ्चिदेवावशेषितम् ||
उन्नम्यमानं क्षतवद्यच्च मज्जति मज्जनि | तद्दुःसाध्यं.....
(*Asṭāṅgahṛdayam – Uttarasthānam* 27.4-6)

prājyāṇudāri yattvasthi sparśe śabdaṃ karoti yat ||
yatrāsthileśaḥ praviśenmadhyamasthno vidāritaḥ |
bhagnaṃ yaccābhighātena kiñcidevāvaśeṣitam ||
unnamyamānaṃ kṣatavadyacca majjati majjani |
taddukhasādhyaṃ.....

3. "When the broken ends of the bone, while being joined, are incorrectly positioned or when bandage is applied improperly or when the (affected) limb is frequently disturbed – if the bone fuses unevenly (in such cases), it is difficult to remedy. If the bone or the bone joints are malformed at birth itself, it is impossible to be cured".

सम्यग्यमितप्यस्थि दुर्न्यासाद्दुर्निबन्धनात् | सङ्क्षोभादपि यद्गच्छेद्विक्रियां तद्विवर्जयेत् ||
आदितो यच्च दुर्जातमस्थि(सन्धिर्न साध्यते)[21] |
(*Asṭāṅgahṛdayam – Uttarasthānam* 27.4-6)

samyagyamitapyasthi durnyāsāddurnibandhanāt |
saṅkṣobhādapi yadgacchedvikriyāṃ tadvivarjayet ||
ādito yacca durjātamasthisandhirna sādhyate |

4. "Immature bones or soft bones tend to bend; hard, long bones tend to break; the broad bones of the skull tend to crack and the rounded bones tend to shatter".

तरुणास्थीनि (नाम्यन्ते भज्यन्ते)[22] नलकानि च | कपालानि (विभिद्यन्ते)[23] स्फुटन्ति रुचकानि च ||

[21] The original Sanskrit text has the words '*sandhirathāpi vā*' instead of '*sandhirna sādhyate*'.
[22] The original Sanskrit text has the words '*namyante bhidyante*' instead of '*nāmyante bhajyante*'.
[23] The original Sanskrit text has the word '*vibhijyante*' instead of '*vibhidyante*'.

(*Mādhavanidānam* 44.12)

tarun◌ाsthīni nāmyante bhajyante nalakāni ca |
kapālāni vibhidyante sphut◌ anti rucakāni ca ||

Please note the meticulousness in defining the bones according to their various shapes and in appropriately naming the different kinds of fractures.

5. Then the methods of aligning the dislocated joints and broken bones and the procedure of applying straps and bandages on them have also been described extensively and lucidly.

6. After aligning the broken or dislocated bones, it is said, "The (affected) limb should be bound with bamboo splints of the right size. The splints should not have any jaggedness and also should not bend easily but should remain stiff; bandages should be first applied before attaching the splints; the splints should also be secured with bandages; the bandages should not be loose; if they are, the bones will not knit properly".

(वंशनाळैश्च)[24] पृथुभिस्तनुभिर्वा (निवेशितैः)[25] | (सुश्लक्ष्णैस्सुप्रतिस्तम्भैर्वस्त्रपट्टोपरिस्थितैः)[26] |
(कुशैर्वापि)[27] समं बन्धं (नालस्योपरि कारयेत्)[28] | शिथिलेन हि बन्धेन (सन्धेः)[29] स्थैर्यं न जायते ||

(*As◌ाngahr◌dayam – Uttarasthānam* 27.15-16)

vam◌śanāl◌aiśca pr◌thubhistanubhirvā niveśitaih◌ |
suślaks◌n◌aissupratistambhairvastrapat◌t◌oparisthitaih◌ ||
kuśairvāpi samam◌ bandham◌ nālasyopari kārayet |
śithilena hi bandhena sandheh◌ sthairyam◌ na jāyate ||

7. There are several instances that show the high level of intelligence and experience of our ancestors in examining and treating fractures. "If the shinbone or the thighbone or the pelvic bone is broken, the special type of bandage called '*kapāt◌aśayanam◌*' is appropriate. The patient should be made to lie down on a plank; to prevent his leg and hip

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[24] The original Sanskrit text has the word '*vam◌śodbhavairvā*' instead of '*vam◌śanāl◌aiśca*'.

[25] The original Sanskrit text has the word '*suniveśitaih◌*' instead of '*vā niveśitaih◌*'.

[26] The original Sanskrit text has the word '*valkalaih◌ śakalairapi*' instead of '*vastrapat◌t◌oparisthitaih◌*'.

[27] The original Sanskrit text has the word '*kuśāhvayaih◌*' instead of '*kuśairvāpi*'

[28] The original Sanskrit text has the words '*pat◌t◌asyopari yojayet*' instead of '*nālasyopari kārayet*'.

[29] The original Sanskrit text has the word '*sandhi*' instead of '*sandheh◌*'.

from moving laterally, two stakes should be fixed on the plank on either side of the shinbone, two stakes on either side of the thigh and one stake under the raised foot for support; the fractured leg should be tightly bound to the five stakes. This type of bandage is called '*kapāt॰aśayanam*'; it is suitable in fractures of the pelvic bone and spine also".

कटीजङ्घोरुभग्नानां कपाटशयनं हितम् ||
यन्त्रणार्थं तथा कीलाः पञ्च कार्या निबन्धनाः | जङ्घोर्वोः पार्श्वयोर्द्वौ द्वौ तल एकश्च कीलकः ||
श्रोण्यां वा पृष्ठवंशे वा भङ्गे चैष विधिर्हितः |
(*As॰t॰āngahr॰dayam – Uttarasthānam* 27.27-29)

kat॰ījanghorubhagnānām॰ kapāt॰aśayanam॰ hitam ||
yantran॰ārtham॰ tathā kīlāh॰ pañca kāryā nibandhanāh॰ |
janghorvoh॰ pārśvayordvau dvau tala ekaśca kīlakah॰ ||
śron॰yām॰ vā pr॰s॰t॰havam॰śe vā bhange cais॰a vidhirhitah॰ |

8. "In cases where dislocated joints have been neglected for a long time and if, in the meantime, tissues develop irregularly and cause problems, oil massages and fomentation should be used to soften the area, after which the physician should employ the procedures listed above or use his discretion to align the joints properly and then apply the bandages as usual. If a fractured bone, which is not part of a joint, knits unevenly by itself and causes asymmetry in the affected limb, the bone should be broken again and immediately set correctly and then bandaged as per procedure".

सन्धींश्चिर(विमुक्तांस्तु)[30] (स्निग्धान् स्विन्नान्)[31] मृदूकृतान् |
उक्तैर्विधानैर्बुद्ध्या (वा)[32] यथास्वं स्थानमानयेत् ||
असन्धिभग्ने रूढे तु विषमोल्बणसाधिते | (पुनरापोथ्य)[33] यमयेत्ततो भग्नवदाचरेत् ||
(*As॰t॰āngahr॰dayam – Uttarasthānam* 27.30-31)

sandhīm॰ściravimuktām॰stu snigdhān svinnān mr॰dūkr॰tān |
uktairvidhānairbuddhyā vā yathāsvam॰ sthānamānayet ||
asandhibhagne rūd॰he tu vis॰amolban॰asādhite |
punarāpothya yamayettato bhagnavadācaret ||

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[30] The original Sanskrit text has the word '*vimuktāstu*' instead of '*vimuktām॰stu*'.

[31] The original Sanskrit text has the word '*snigdhasvinnān*' instead of the words '*snigdhān svinnān*'.

[32] The original Sanskrit text has the word '*ca*' instead of '*vā*'.

[33] The original Sanskrit text has the words '*āpothya bhagnam॰*' instead of the word '*punarāpothya*'.

When Vāgbhat◌̃a has given such clear and practical explanations in the *As◌̃t◌̃āngahr◌̃dayam*, what reply can I give to those who opine that surgery is a new field in Āyurveda and that we have to borrow the methods from western physicians?

Methods of Bandaging

Just as triangular bandages, T-bandages, etc. are known to be competently used by western physicians for wounds and fractures, the detailed explanations in our texts show that our medical system practises many more techniques of bandaging. Fifteen types of bandages such as *kośa*, *svastika*, *muttolī*, *cīnam*, *dāman*, *anuvellitam*, *khat◌̃vā*, *vibandha*, *sthagikā*, *vitānam*, *utsanga*, *gophan◌̃ā*, *yamakam*, *man◌̃d◌̃alam* and *pañcāngī*, named according to their shapes, have been described, together with the body parts for which they are suitable. Besides, the physicians are also required to know that there are three levels while applying them – tight, loose or intermediate. Items such as lint, bandage and cloth that are in use today are called '*vikeśikā*', '*kavalikā*' and '*pat◌̃t◌̃abandhanam*' in Āyurvedic texts, which also give their dimensions.

Specialised Surgery

It is said that the western physicians learnt from the Āyurvedic system the method of rhinoplastic surgery, which involves repairing the damaged part of the nose by attaching the topmost layer of skin from the cheek (as mentioned in the *As◌̃t◌̃āngahr◌̃dayam – Uttarasthānam – Nāsārogapratis◌̃edhādhyāya*). The specialised surgeries that form part of the treatment of ailments above the collar-bone include the '*śālākyacikitsā*'. I have already mentioned that surgery done on the eye, ear and nose are known as '*śālākya*'. There were several texts written on these subjects, titled '*Videhatantram*', '*Nimitantram*' and '*Caks◌̃us◌̃yatantram*', which are not extant now.

These days, there is a proliferation of dentists everywhere. Why cannot at least a few of the Āyurvedic doctors learn this field (dentistry) and diligently practise it? The *Suśrutasam◌̣hitā* says, "If a tooth develops a cavity due to decay, it should be extracted; the affected area should be cleaned and cauterised with caustic salt; even if there is a fleshy growth, it should be cauterised with caustic salt and cleaned; if neglected, this condition will lead to much problem, with the decay spreading to the jawbone below, resulting in harm; ignoring a half-broken tooth will cause pain, besides resulting in other problems. Hence the tooth should be completely extracted with pincers".

(यस्मिन् दन्तेऽभिजायेत)[34] नाडी तं दन्तमुद्धरेत् | शोधयित्वा दहेद्वापि (क्षारैस्तत्राधिमांसकम्)[35] ||
भिनत्त्युपेक्षिते दन्ते हनुकास्थिगतिर्ध्रुवम् | (नोत्तुण्डितमुपेक्षेत स चोपद्रवमावहेत्)[36] ||
समूलं दशनं तस्मात् (सन्दंशेनोद्धरेद्बलात्)[37] |

(*Suśrutasaṁhitā – Cikitsitasthānam* 21.27-29)

yasmin dante'bhijāyeta nāḍī taṁ dantamuddharet |
śodhayitvā dahedvāpi kṣāraistatrādhimāṁsakam ||
bhinattyupekṣite dante hanukāsthigatirdhruvam |
nottuṇḍitamupekṣeta sa copadravamāvahet ||
samūlaṁ daśanaṁ tasmāt sandaṁśenoddharedbalāt |

In addition, the procedure of venesection for bloodletting, the differences among the leeches and the ways of applying the healthy one to draw out blood from the diseased area and the methods of treating tumour or hyperaemia using chalk can be learnt by studying Āyurveda. Therefore, there should be no hesitation in saying that Āyurveda is a complete medical system, which includes surgery and all required components. It is true that we have not had the practice (in surgery) for some time. But it is unfair to argue that the system is incomplete when we have not taken the effort to train ourselves.

Obstetrics

If we analyse the branch of obstetrics, we see that it is also found to be complete in our medical system. "In the ninth month itself, the pregnant woman should be taken to the lying-in chamber. As the time of delivery approaches, her body will feel drained; her face will have a languorous expression; her eyes will be sunken; she will feel as if her breasts are drooping; her upper abdomen will sag; her lower abdomen will seem heavy; the pain will start at her hips and groins; then the amniotic fluid will flow out; she will lose taste for food and then the actual contractions will begin. At this stage, she should be made to lie on her back on a bed having a soft, smooth pillow and should be attended to by four women; these women should be adept in midwifery, free from doubts, clean and should have trimmed nails".

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[34] The main Sanskrit text has the words '*yaṁ dantamadhijaayeta*' instead of '*yasmin dante'bhijāyeta*'.

[35] The Sanskrit text has the words '*kṣāren a jvalanena vā*' instead of '*kṣāraistatrādhimāṁsakam*'.

[36] This line is not found in the original Sanskrit text.

[37] The Sanskrit text has the words '*uddharedbhagnamasthi ca*' instead of '*sandaṁśenoddharedbalāt*'.

नवमे मासि सूतिकागारमेनाम् (आनयेत्)[38]

(*Suśrutasam◌ीhitā – Śārīrasthānam* 10.5)

navame māsi sūtikāgāramenām ānayet

(प्रजननकाले)[39] क्लमो गात्राणां ग्लानिराननस्य अक्ष्णोश्शैथिल्यं विमुक्तबन्धनत्वमिव वक्षसः कुक्षेरवस्रंसनम् अधोगुरुत्वं वङ्क्षणवसतिकटीपार्श्वनिस्तोदः योनेः प्रस्रवणम् अनन्नाभिलाषश्चेति ततोऽनन्तरमावीनां प्रादुर्भावः प्रसेकश्च गर्भोदकस्य |

(*Carakasam◌ीhitā – Śārīrasthānam* 8.36)

prajananakāle klamo gātrān◌ाम् glānirānanasya aks◌ान◌ाośśaithilyam◌ी vimuktabandhanatvamiva vaks◌ाasah◌ा kuks◌ाeravasram◌ीsanam adhogurutvam◌ी vanks◌ाan◌ाavasatikat◌ाīpārśvanistodah◌ा yoneh◌ा prasravan◌ाam anannābhilās◌ाaśceti tato'nantaramāvīnām◌ी prādurbhāvah◌ा prasekaśca garbhodakasya |

(तां)[40] कृतोपधाने मृदुविस्तीर्णे शयने स्थिताम् ..... उत्तानाम् (अवदाताः)[41] अशङ्कनीयाः चतस्रस्त्रियः ..... प्रजननकुशलाः कर्तितनखाः परिचरेयुः |

(*Suśrutasam◌ीhitā – Śārīrasthānam* 10.8)

tām◌ी kr◌ृtopadhāne mr◌ृduvistīrn◌ाe śayane sthitām ..... uttānām avadātāh◌ा aśankanīyāh◌ा catasra- striyah◌ा ..... prajananakuśalāh◌ा kartitanakhāh◌ा paricareyuh◌ा |

After further describing the medical treatments and the assistance of the midwife that are required during a normal delivery, details are provided about obstructed labour or stillbirth, the various difficulties therein and the surgeries done using instruments, which have already been mentioned earlier. The remedial measures and methods to be followed if the placenta is not expelled after delivery, the various treatments for post-natal disorders in the mother have all been exhaustively explained in the texts of Suśruta and Vāgbhat◌ाa.

Child-rearing

The only medical system that is timeless and complete in all areas is Āyurveda. It gives us full instructions on how to bring up a child. The practice of giving bottled milk and artificial milk to babies has been proscribed not just by us, but also strongly and repeatedly by the modern western physicians, whose words are taken as decrees in today's world. But it is only

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[38] The original Sanskrit text has the word '*praveśayet*' instead of '*ānayet*'.

[39] The original Sanskrit text has the words '*prajananakālamabhito bhavanti tadyathā*' instead of '*prajananakāle*'.

[40] The original Sanskrit text has the word '*tatah◌ा*' instead of '*tām◌ी*'.

[41] This word is not found in the original Sanskrit text.

this artificial milk – made in other countries and arriving here in shiploads – that is given as the 'latest and best food' to babies by parents who are enamoured of the western culture.

मातुरेव पिबेत्स्तन्यं तत्परं देहवृद्धये | (वत्सलायाः पयो धात्र्यास्तदभावे तु पाययेत्)[42] ||
(*Aṣṭāṅgahṛdayam – Uttarasthānam* 1.15)

mātureva pibetstanyaṁ tatparaṁ dehavṛddhaye |
vatsalāyāḥ payo dhātryāstadabhāve tu pāyayet ||

"Infants should feed only on mothers' milk; that alone is the best source of nourishment. In its absence, the infant may be nursed by an affectionate and caring wet-nurse."

If Āyurveda contains this injunction, should we not consider it equal to the divine scriptures? Should we not totally abandon our distrust of Āyurveda as well as the evil practice of feeding bottled milk, etc. to our children?

The required rules for a wet-nurse are also given: "A wet-nurse should have living children, should be free from diseases, hygienic and given to eating nutritious food".

जीवद्वत्सा नीरुजा च शुद्धा धात्री हिताशिनी[43]

jīvadvatsā nīrujā ca śuddhā dhātrī hitāśinī

A convenient replacement for breast-milk is also given: "If mother's milk is not available, then the infant can drink either goat's milk or cow's milk, both of which are almost equal to breast-milk". We know that western physicians also wholeheartedly recommend this method.

स्तन्याभावे पयश्छागं गव्यं वा तद्गुणं पिबेत्
(*Aṣṭāṅgahṛdayam – Uttarasthānam* 1.20)

stanyābhāve payaśchāgaṁ gavyaṁ vā tadguṇaṁ pibet

The chapter titled '*Kaumārabhṛtya*' of the *Aṣṭāṅgahṛdayam* contains clear and complete details about the steps to be followed while putting an infant to bed or when the baby is teething or attempting to speak or trying to walk as well as the remedial measures for various ailments in infants.

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[42] This line is not found in the original Sanksrit text.

[43] This line is not traceable. But the *Aṣṭāṅgahṛdayam – Uttarasthānam* 1.16 has a similar meaning.

“Their (the infants’) beds and clothes should be clean, washed daily, without any unevenness or wrinkles, very soft and fumigated with medicated smoke.”

शुचिधौतोपधानानि निर्वलीनि मृदूनि च | शय्यास्तरणवासांसि रक्षोघ्नैर्धूपितानि च ||

(*Astāṅgahṛdayam – Uttarasthānam* 1.25)

śucidhautopadhānāni nirvalīni mṛdūni ca |
śayyāstaraṇavāsāṁsi rakṣoghnairdhūpitāni ca ||

Is this ancient injunction inferior to the modern method in efficacy?

Caraka advises, “No action should be done which induces fear in infants”.

न ह्यस्य वित्रासनं साधु |

(*Carakasaṁhitā – Śārīrasthānam* 8.64)

na hyasya vitrāsanaṁ sādhu

“They should be entertained with pretty and musical toys; those toys should neither be sharp nor heavy”.

क्रीडनकानि खलु कुमारस्य विचित्राणि घोषवन्त्यभिरामाणि चागुरूणि चातीक्ष्णाग्राणि.....

(*Ibid*. 8.63)

krīḍanakāni khalu kumārasya vicitrāṇi ghoṣavantyabhirāmāṇi cāgurūṇi cātīkṣṇāgrāṇi....

अथैनं जातदशनं कर्मणोपनयेत् स्तनात्

(*Astāṅgahṛdayam – Uttarasthānam* 1.37)

athainaṁ jātadaśanaṁ karmaṇopanayet stanāt

“The infant should be weaned after teething.”

अपस्तन्यस्य संयोज्यः प्रीणनो मोदकः शिशोः

(*Ibid*. 1.38)

apastanyasya saṁyojyaḥ prīṇano modakaḥ śiśoḥ

“After the infant stops taking breast-milk, he should be fed sweet food items.”

The above recommendations regarding teething are now being made by western physicians to their patients as though these are new techniques.

We know that our womenfolk administer castor oil as a laxative to their young children almost everyday. Not just that, I have seen some ignorant women give strong purgatives such as herbal oils (called '*mānda enॅnॅey*' or '*panॅdॅāra-paradeśi enॅnॅey*' in Tamil) and tablets (known as '*sañjīvi*' tablets) to their children everyday. That this practice of giving such aperients, and that too strong ones, to children everyday is extremely harmful is the opinion of modern physicians as well. When I firmly told several mothers that laxatives should not be given to children often, they disparagingly replied, "This is what present-day physicians will say", thus putting me also in the class of western physicians. However, the injunction contained in Āyurvedic texts about frequent purgation for children is very ancient.

अन्यत्रात्ययिकाद्व्याधेर्विरेकं सुतरां त्यजेत्

(*Ibid*. 1.40)

anyatrātyayikādvyādhervirekam॰ sutarām॰ tyajet

"Laxative should never be given to children except when absolutely necessary in acute sickness."

"Teething in babies is the cause of all diseases; it specially brings on fever, diarrhoea, cough, vomiting and headache. The procedure of treatment is to first examine the affected humour, diagnose the disease, ascertain the main one among the many disorders, find out which part of the body is affected as well as the place and time before prescribing a remedy."

दन्तोद्भेदश्च रोगाणां सर्वेषामपि कारणम् ||
विशेषाज्ज्वरविड्भेदकासच्छर्दिशिरोरुजाम् |
यथादोषं यथारोगं यथोद्रेकं यथाशयम् | विभज्य देशकालादींस्तत्र योज्यं भिषग्जितम् ||

(*Ibid*. 2.26,27 and 29)

dandodbhedaśca rogānॅām॰ sarvesॅāmapi kāranॅam ||
viśesॅājjvaravidॅbhedakāsacchardiśirorujām |
yathādosॅam॰ yathārogam॰ yathodrekam॰ yathāśayam |
vibhajya deśakālādīm॰statra yojyam॰ bhisॅagjitam ||

Besides these, several other ways of treating children in health and sickness are lucidly described in the *Uttarasthānam* of the *Asॅtॅāngahrॅdayam*.

Pharmacology

The terms '*Dravyagun◌̃apāt◌̃ha*' or '*Os◌̃adhinighan◌̃t◌̃u*', which refer to a glossary of Āyurvedic medicines, may remind Allopathic doctors of the *Materia Medica*. The *Os◌̃adhinighan◌̃t◌̃u*, which is based on the principle of the *tridos◌̃as*, is naturally bound to be different from the *Materia Medica* in many ways. The composers of Āyurvedic lexicons have refrained from using riddles or unnecessary words and have clearly explained the characteristics and nature of each element in their works. While the section titled *Sūtrasthānam* in the works of Caraka and Vāgbhat◌̃a briefly covers these principles, the *Rājanighan◌̃t◌̃u*, the *Dhanvantarinighan◌̃t◌̃u*, the *Madanapālanighan◌̃t◌̃u*, the *Br◌̃hannighan◌̃t◌̃u*, the *Rasaratnākara*, etc. contain detailed and classified information about these fundamental features.

The medicinal ingredients extracted from *sthāvara-jīvavarga* (stationary beings) like plants, creepers, etc. and those derived from the bodies of birds, animals, etc. are grouped under '*sendriya-vastu*' and those obtained from minerals, metals and ores are known as '*nirindriya-vastu*'. Beings that are endowed with consciousness are called '*sendriya*', while '*nirindriya*' means 'without consciousness'.

सेन्द्रियं चेतनं द्रव्यं निरिन्द्रियमचेतनम्

(*Carakasam◌̀hitā – Sūtrasthānam* 1.48)

sendriyam◌̀ cetanam◌̀ dravyam◌̀ nirindriyamacetanam

This is how Caraka has classified the Āyurvedic drugs. The chemists of the western system of medicine use the terms 'organic and inorganic substances', which is a rough translation of the above names.

The composers of the Āyurvedic lexicons have specified the seasons and the locations for obtaining herbs and also the parts of the plants that should be used, besides providing reasons for their recommendations. Another speciality of these lexicons is that they contain detailed information about the taste of the ingredient such as sweet, sour, etc., their secondary flavours, their potencies such as hot, cold, etc. and the change in *rasa* that occurs after they reach the intestine. Furthermore, the effect of the drugs, the twenty varieties of their nature, *viz*. heaviness, lightness, oiliness, astringency, etc., as well as their functions and the diseases that they help to cure have been explained, all of which are exceedingly useful to physicians. These texts – which are based on the principle of *tridos◌̃a* and which mention the qualities and properties (of the organic and inorganic substances mentioned earlier) – contain some unusual concepts that are not seen in western medicine. But it should be said that the details of properties or specific actions (of drugs) mentioned in the western medical system are

definitely covered much more extensively in Āyurvedic lexicons. Some of the technical terms used in these texts to describe the properties of drugs are[44]:

- '*dīpanam*' (appetiser – they increase the digestive fire, but do not digest undigested food)
- '*pācanam*' (digestives – they digest undigested food, but do not increase the digestive fire)
- '*śamanam*' (palliatives – they pacify the aggravated elements, but do not expel them or increase the balanced elements)
- '*anulomanam*' (carminatives – they help to assimilate food, break down the obstructed impurities and bring it downward)
- '*sraṁsanam*' (laxatives – they do not take part in digestion, but drive down the impacted digestive by-products that remain in the intestines)
- '*bhedanam*' (purgatives – they break down the excreta and push it downwards)
- '*recanam*' (strong purgatives – they liquefy the impurities, whether digested or undigested, and expel them from the intestines)
- '*vamanam*' (emetics – they forcefully bring undigested bile and phlegm and expel them through the mouth)
- '*deha-saṁśodhanam*' (cleansers – they remove the impurities accumulated after digestion, either through the mouth or the rectum)
- '*chedanam*' (they expel not just the *dos̃as*, but also other (unnecessary / harmful) elements such as overgrown tissues in the body)
- '*lekhanam*' (they scrape out the impurities by causing extreme dryness in the humours and tissues of the body)
- '*grāhī*' (they help in the absorption of excessive moisture in the intestinal mucosa due to their heat-generating property; they have the properties of both *dīpanam* and *pācanam* drugs)
- '*stambhanam*' (they cause stasis and condensation of the fluids by their cooling, drying and astringent properties; they are also easily digestible)
- '*rasāyanam*' (restorers / vitalisers – they help in preventing diseases and postponing the symptoms of old age)
- '*vājīkarañam*' (aphrodisiacs – they increase sexual desire and eroticism)
- '*ārtavajanakam*' (emmenagogues – they induce menstruation)
- '*ārtavastambhanam*' (they block the menstrual flow)
- '*sūks̃mam*' (transdermal drugs – it enters the body directly through the pores)
- '*vyavāyī*' (drugs that spread throughout the body before being digested)
- '*vikāśī*' (it deprives the seven tissues of their vigour and causes looseness in the joints and related parts of the body)
- '*madakārī*' (intoxicants – they cause disturbance in the person's intellectual faculties)

[44] The meanings are taken from the *Śārngadharasaṁhitā – Pūrvakhan̄d̄a*, chapter 4

- '*yogavāhī*' (synergistic drugs – they increase the availability and efficacy of the active drugs with which they are mixed)
- '*pramāthī*' (they remove the accumulated phlegm etc. from the nasal and tympanic cavity)
- '*abhis*◌̃*yandī*' (they cause obstruction in the capillaries due to their stickiness and heaviness and cause heaviness in the body)
- '*vedanāsthāpanam*' (anodynes / analgesics – they relieve pain)
- '*śon*◌̃*itāsthāpanam*' (styptics / haemostatics – they control bleeding by contracting the blood vessels)
- '*śon*◌̃*itavrajanīyam*' (they restore the normal colour of blood)
- '*medhyam*' (nootropics – they improve cognitive functions)
- '*balyam*' (tonics – they increase strength and vitality)
- '*hr*◌̽*dayam*' (cardiotonics – they increase the efficiency and improve the contraction of the heart muscles)
- '*tvacyam*' (unguents – they provide moisture and overall nourishment for the skin)

Pharmaceutics

This branch of medicine, which is called '*aus*◌̃*adhayoga*' in Sanskrit, is commonly known as '*Vākat*◌̃*avidhi*' in Tamilnadu. There is a reason behind this (Tamil) name. All of us now know that the author of the *As*◌̃*t*◌̃*āngahr*◌̽*dayam* and the *As*◌̃*t*◌̃*āngasangraha* is Vāgbhat◌̃a. He is also known as 'Bāhat◌̃a'. Based on the principle of '*vabayorabheda*' (non-difference between the letters '*va*' and '*ba*'), the word 'Bāhat◌̃a' is modified into 'Vākat◌̃a'. Since the *As*◌̃*t*◌̃*āngahr*◌̽*dayam* and the *Rasaratnasamuccaya* are written by Vākat◌̃a and since these two works are well known for long in the field of pharmaceutics, the section dealing with the formulation of medicines is itself known as '*Vākat*◌̃*anūl*' or '*Vākat*◌̃*avidhi*' (in Tamil). This science, which is known as 'pharmacy' in western medicine, involves weighing and measuring the ingredients and methods of using them while still fresh and when dried. This branch of medicine describes in detail the methods of preparation of '*pañcakas*◌̃*āyas*' (the five types of potions), *viz*. *svarasa* (fresh extracts), *kalka* (pastes), *kvātha* (decoctions, also called '*kas*◌̃*āya*'), *phān*◌̃*t*◌̃*a* (hot infusions) and *hima* (cold infusions) as well as several preparations like *cūrn*◌̃*am* (powders), *gulikā* (pills), *vat*◌̃*akam* (tablets), *modakam* (globules), *lehyam* (electuaries), *rasakriyā* (*kvātha*, *svarasa*, *phan*◌̃*t*◌̃*a* and *hima* boiled over low fire and reduced to a semisolid consistency), *tailam* (medicated oils), *ghr*◌̽*tam* (medicated ghee), *āsava* (naturally-fermented fresh herbal juices), *aris*◌̃*t*◌̃*a* (naturally-fermented decoctions), *arka* (distillations), *añjanam* (eyewash), *lepam* (salves), etc. Of these, *gulikā*, *vat*◌̃*akam* and *modakam* are round or flat in shape and small in size for easy ingestion and hence go by the general name of '*mātrā*' (meaning 'dosage'). The two

liquid formulations of *āsava* and *aris*◌̃*t*◌̃*a* are prepared by storing the specific infusions in vats and burying them in grains or special pits, which causes them to ferment. This method of preparation is called '*sandhānam*'. Since *tailam* and *ghr*◌̃*tam* are generally oil-based or fat-based, their method of preparation is known as '*snehapākam*'. *Arkas* are liquid medicines that are made by distillation process using a condensation device. A kind of liquor available today, which is called 'arrack', is also prepared by this method and then allowed to ferment. The word 'arrack' is nothing but a modification of the word '*arka*'.

As Ācārya Vāgbhat◌̃a has explained the necessary methods of treatment and remedies of almost all diseases in the *Cikitsāsthānam*, *Kalpasthānam* and especially in the *Uttarasthānam* of his *As*◌̃*t*◌̃*āngahr*◌̃*dayam*, the book is well known as '*Vākat*◌̃*a*', besides being very useful in the state of Kerala. Many Malayālīs memorise and recite the verses of the *As*◌̃*t*◌̃*āngahr*◌̃*dayam* like a hymn. In fact, for the aesthetes and scholars, the verses of the *As*◌̃*t*◌̃*āngahr*◌̃*dayam* are as enjoyable as poetry, while for physicians, both old and young, it is a rare and wonderful storehouse of knowledge.

Another remarkable medical text which contains the descriptions of the formulations such as decoctions, powders, electuaries, etc., mentioned earlier, and provides a sequential list of the most well-known, effective and important methods of preparation of each formulation in a way that is very useful for a hospital or a physician is the '*Śārngadharasam*◌̣*hitā*'. Details of diseases and the technical terms are given in the first part, while the instructions for preparing all drugs are found in the second part and the procedures of treatments such as *sneha-sveda-vamana-recana* are described in the third part, thus presenting all topics in these three parts of this novel compilation, which was authored by Śārngadhara. Knowing that it is a unique medical text that would be useful to all in several ways, I translated and published it first to help our brethren in Tamilnadu. Since no system other than Āyurveda has mentioned the fermentative processes of preparing *āsava* and *aris*◌̃*t*◌̃*a* and the applications of *rasakriyā*, it must be concluded that in the branch of pharmaceutics also, the system of Āyurveda is superior to and much more useful than all other medical systems.

Books on Metallurgy

I have explained at the beginning that the preceptors of Āyurveda were eminently skilled in the use of minerals, metals, rocks and ores as medicines. This knowledge, it is seen, enabled them to attain the pinnacle of perfection in preparing drugs from *sendriya* ingredients (like plants, creepers, etc.) and processing of medicines from *nirīndriya* elements (like metals, etc.) in those days (*i.e.* even during the time of the sage Patañjali). Realising that there is usually a marked difference in potency between the medicines prepared from herbs and those extracted from minerals, metals, etc., they have announced their findings through the following verse.

(स्वल्प)[45]मात्रोपयोगित्वादरुचेरप्रसङ्गतः | (अचिराद्बल)[46]दायित्वादोषदीभ्योऽधिको रसः ||
(*Rasaratnasamuccaya* 28.1)

svalpamātropayogitvādarucerāprasangatah |
acirādbaladāyitvādos *adībhyo'dhiko rasah* ||

(As mineral remedies are effective even in small doses, are not unpalatable and bestow stamina immediately, they are superior to herbal remedies.)

Among the ancient works on metallurgy, the *Rasaratnasamuccaya* of Vāgbhat a is famous throughout this country. Even Dr. Sir P C Ray has stated that he mostly followed this work while writing his book, '*History of Hindu Chemistry*'. Therefore, I have, with great effort, translated and published this work also, which is known to many in Tamilnadu.

Before the 15th century, no one in Europe thought of the possibility of using minerals, etc. as medicines. In 1566, the members of the Faculty of Medicine in Paris, not knowing this fact (that drugs could be extracted from minerals, metals, etc.), had warned against their use. In the beginning of the 19th century, Ainslie (Dr. Whitelaw Ainslie) writes in his book '*Lepra Arabum*' that it was the Indians who knew long ago that many skin disorders, especially diabetes and leprosy as well as other serious diseases, could be treated with minerals, metals, rocks, etc., which method they employed.

Need for Experience in Surgery

Now the only requirement is that Āyurvedic physicians should be experienced in surgery. I have already established that we are fully competent in all other fields and that we have almost complete technical details in the branch of surgery. The Encyclopaedia Britannica contains specific information that surgery was done in India in the past, that this science spread from here to the western countries and that the apparatuses and instruments used in the Indian medical system had admirably increased in number. Hence, knowing that we have the right to acquire the necessary experience in surgery – and not the basic information (which we already possess) – Āyurvedic physicians may strive to learn the skill from the western medical system.

Astuteness of Westerners

I am not at all criticizing either the westerners or their medical system. Keeping the medical techniques that they had learnt from here as their basis, they then improved upon it. So,

[45] The original Sanskrit text has the word '*alpa*' instead of '*svalpa*'.

[46] The original Sanskrit text has the word '*ks ipramārogya*' instead of '*acirādbala*'.

besides finding ways to improve the health and wealth of their citizens, they are also serving the country lovingly and carefully, since they are concerned with its welfare. They have given form, structure and gloss to the knowledge gained or things taken from here, thus completely transforming them. Using these products (and ideas) commercially, they enthusiastically and very cleverly find ways to increase their country's wealth and influence. Despite seeing this time and again for almost two hundred years, we continue to remain thoughtlessly idle.

The westerners buy bales of cotton cheaply from here, convert them into delicate threads or beautiful garments in their country and sell them back to us, earning a hundredfold profit. Similarly they purchase oilseeds from our country, manufacture fancy soaps and candles and send them here, amassing great wealth in the process. They also buy natural herbs, spices and other items at a low price, remodel them to look like new medicines, pack them in attractive bottles and boxes and employ several ingenious means to sell them to us through our own Allopathic brothers. All these resourceful efforts on their part only show their extraordinary concern for their country and their unflagging persistence.

Just as they take away these natural assets, embellish them artificially, sell them back to us and earn a huge profit, they also learn our medical system – which we consider unimportant – subject it to fake reforms and claim that it is a new western system. Using our Indian brothers trained in western medical system as their instruments, they spread their revamped methods throughout our country, thus further increasing their wealth and authority. In spite of seeing this directly, we choose to remain blind and lackadaisical. We do not consider our natural resources as valuable; but the westerners eagerly take them, make them valuable and benefit from them. We hand over our nature-based medical system to them and they refine and use it. Seeing that we are naïve people, they use us as well to their advantage. So it is the wish of discerning individuals among us that, even though they fail to impress us, we should first zealously protect and hone our natural resources and the nature-based medical system devised by our ancestors and then, besides proving that we are cultured people right from early times, we should also augment our wealth.

Unrighteousness of Neglecting Āyurveda

From the points that I have mentioned so far, together with suitable evidence, it would be clear to the audience where we are heading. The system of Āyurveda is ageless and eternal. Unlike other medical systems which keep changing every day, it is an authentic science with an excellent foundation. It has been an exalted system, complete with all divisions and subdivisions, including surgery. For the last couple of centuries, due to lack of governmental support for various reasons and our own disregard for the practice of surgery, we opine that Āyurveda in general, and surgery in particular, has slipped from its lofty position. As Sir P Ramanathan has very insightfully pointed out at the Colombo Conference, right from the day

the British government came to power, people have gradually become infatuated by the western customs and habits; the medical department has been established on the lines of other departments under the rule of the British; the British businessmen who control the department have imported doctors, medicines and many apparatuses from western countries, thus causing the neglect of our medical system and pushing it from its earlier high position to being termed worthless. All these are being witnessed by our guileless public.

Have not the works composed by our hallowed ancestors helped us, until now, to cure diseases, enable the people to gain physical strength and the country to become prosperous? Will it not wring the hearts of noble patriots to see that the very people born in the sacred lineage of those great sages are complicit in uprooting and utterly destroying this system, which is based on the scriptures and which was the source of the western medical system?

Most of us do not consider it as a matter of pride that even today, each of us still mention as our *gotram* the name of the sage who was our ancestor and that this is a unique privilege not available to any other race. Every one of us can know without a shadow of doubt that, since we go by the name of our *gotram*, every descendant in that particular lineage has pure blood flowing in their bodies, right from ancient times. Who knows how many thousands of years ago Sage Ātreya, the forebear of the *Ātreya-gotram*, lived? Even though we do not know the period of Sage Kaśyapa, those born in the clan of that sage are spread throughout India. We are not born in families which has meaningless names such as ‘Tithan’, ‘Dipiton’, ‘Box’ and ‘Cook’ and whose ancestries cannot be traced.

If we strongly and justifiably feel that each of us, born in the noble families comparable to necklaces of exquisite pearls, is like a flawless pearl in that necklace and a gem in the spiritual lineage of those divine primogenitors, is it not a crime on our part to bring about the destruction of the sacred system of Āyurveda, which was assiduously established and handed to us by our godly ancestors? This callousness, which goes by the name of ‘new culture’, has arrived to sound the death knell for our hoary system of Āyurveda, just as it is doing for our sacred religious systems.

Now, this assembly has gathered here to request everyone to support the system of Āyurveda – which was bequeathed to us by our wise ancestors and which confers good health as well as the three objects of human pursuit, *viz*. righteousness, prosperity and fulfilment of desires – so that no harm befalls it. While the missionaries, without knowing the doctrine of spirituality, (but) with full conviction and boundless regard for their religion, collected funds through the public and the government, worked tirelessly, constructed churches at several places, relentlessly preached their religion, printed attractive copies of their books, freely distributed them and exerted themselves in all ways to achieve their objective, did not everybody, right from the Governor to the labourers, support these worthless efforts of the

missionaries and make the campaign successful? Do you think that Āyurveda is inferior to this (campaign)? It was extremely saddening to read the Secretary's note to me mentioning the difficulty in finding donors for this hallowed conference.

Veneration for Indigenous Science

Everything will seem fair if there is genuine regard. If there are adherents, Āyurveda can be revealed to the world as an extraordinary science. The westerners, who are looked upon as the vanguards of modern culture, normally consider it their duty to adequately support and respect every scheme of their country – be it religious or worldly or anything else.

Not many of us would have carefully gone through the recent annual report of the Department of Medicine published by the Surgeon General of Bengal (G F A Harris), wherein he claims that the Government does not provide the facilities that were given earlier to the officers of Indian Medical Service; that it is only these officers who would be very useful to the country; that increasing the number of Indians (in the IMS) will result in a decline in the medical system. He also declares that his heart melts on seeing the sufferings of the citizens of this country. This is like a wolf expressing its concern for the lambs!
The Surgeon General further states that people come for treatment only to the city hospital, not to those in the outer districts; the reason is that majority of the people trust the Āyurveda and Unāni systems of medicines and not Allopathy. He also says that if additional expenses are incurred to develop Allopathy in those areas, people will no longer prefer Āyurveda.

His report thus reveals his utmost faith in his system of medicine as well as in the doctors of his country and his intentions to wipe out our native medical systems.

Despite the fact that Indian doctors have studied western medicine and have very capably cured patients, the enthusiasm of the British in acknowledging their skill automatically decreases the moment it occurs to them that these doctors are Indians. When loyalty to the nation exists so strongly in the westerners, who are the mentors of modern culture, should not our brethren, particularly those keen on this way of life, emulate the westerners and vow to support our system of medicine and our doctors? There is no point in saying that our medical system has some shortcomings. I have already explained that it is complete. Besides, it is rare to find perfection in the world. Even if there are (deficiencies), the following words of Caraka provide the opportunity to rectify them: "Whatever gives relief to the sick may be approved and used by a proficient doctor who is desirous of advancing in his profession".

यद्यत्सुखाय चार्तानां व्यादिभिः परिदृश्यते | भिषजा तत्तदादेयं निपुणेन बुभूषुणा ||[47]

[47] Source is not available for this verse.

yadyatsukhāya cārtānāṁ vyādibhiḥ paridṛśyate |
bhiṣajā tattadādeyaṁ nipuṇena bubhūṣuṇā ||

Royal National Institute of Medicine

As a result of the strong propaganda and protests carried out by us for the last twenty years, the Usman Committee was formed in Chennai. Our friend Vaidyaratnam G. Srinivasamurti, who was appointed as its Secretary, wrote a very powerful report and sent it to the Government, which then established the National Medical College. Brushing aside the lack of enthusiasm due to the diminished strength of the Indian medicine and the doctors on one side and the strong bombardment of the western medical system and its doctors on the other, getting the Government to set up the National Medical College and administering it without hurdles is not an easy task. Besides having to establish two separate sections for Āyurveda and Unāni systems, he also had to prepare the syllabus for the new scheme called 'Siddha', which had appeared like 'a boil on an ulcer' (*Abhijñānaśākuntalam*, Act II: '*gaṇḍasyopari piṭakā samvṛttā*'). I know of the responsibility and effort he had undertaken in ensuring that, despite several obstacles, these departments function under one roof without any conflict and also in managing the entire institute. We should be pleased that God has given him the ability and patience to run such an organisation. In addition, if he also incorporates a few reforms suggested by us to the extent possible, and without considering them as nuisance, it will help in the development of our indigenous system of medicine.

1. Firstly, the syllabus and teachers should be such that they give preference and importance to the practice of our medical science. As I have already said until now and as Śrīmān Srinivasamurti has written in the Usman Committee report, the only aspect that should be learnt by our doctors from the western counterparts is experience in surgery. Hence, changes should be made so that our native medical system remains prominent which, I feel, will not be too difficult.

2. Many feel that not sufficient books have been designed for L.I.M (Licentiate in Indian Medicine) classes. The complete *Aṣṭāṅgahṛdayam*, the *Cikitsāsthānam* and a few other sections of the *Caraka saṁhitā*, the *Śārīrasthānam* and a few other chapters of the *Suśruta saṁhitā*, the full text of the *Mādhavanidānam*, the *Dhanvantarinighaṇṭu* or the *Rājanighaṇṭu*, the *Rasaratnasamuccaya* and the *Nāḍīvijñānam* should be streamlined and included in the syllabus. In the first year, the *Śārngadharasaṁhitā*, the *Nāḍīvijñānam*, the *Sūtrasthānam* of the *Aṣṭāṅgahṛdayam* and the *Nighaṇṭu* may be taught. When done this way, the L.I.M class will be strong in Āyurveda. The subjects that I have mentioned for a four-year course are very moderate.

3. Since this National Institute of Medicine also includes hospitals, medicines are being prepared for their use. As the local board and municipal hospitals too have to mostly use only these drugs, a revision of the list of medicines will be convenient in many ways. According to the list, around 225 medicines are required for the hospitals. The first point is to decide whether free dispensaries need that many medicines; the other would be to refine the procedure of selection, which will be advantageous.

4. The registration of national physicians is a topic discussed by physicians all over. When a bill was passed in 1913 to register the practitioners of western medicine, Pan͡d͡it Gopālācārlu, I and a few others have jointly protested that such a bill was essential for the eligible doctors of Āyurveda and Unāni as well. I recollect the statement given in the same year by Sir Purday Lugis[48], a witness at the Public Service Commission: "These days, people with no eligibility call themselves doctors, creating havoc. To prevent this, a law should be enacted in all presidencies, just as it was done in Bombay". Caraka says, "Only due to the neglect of the king, bad doctors go about causing harm to the people" (*Carakasam̐hitā – Sūtrasthānam* 29.8: '*bhis͡akchadmapraticchannāh͡ kan͡t͡akabhūtā lokasya ..... rājñām̐ pramādāccaranti rās͡t͡res͡u*[49]'). Suśruta also echoes this sentiment. The registration process should be implemented without adversely affecting doctors who are skilful, knowledgeable and experienced.

5. Separate arrangements should be made in the college for research in the field of traditional medicine.

Some Evils in Modern Culture

This assembly has gathered for the intention of developing the science of Āyurveda – which is the basis for providing mental and physical wellbeing – and to arrange for the welfare of the practitioners of this science. If even a few of the attendees who have heard these speeches go back home without any involvement in the subsequent tasks, this meeting will end up as a futile exercise. It is vital to focus on a modest lifestyle. In the current situation, where the world in general and India in particular, especially South India, is battling severe economic difficulties, our people, specifically our youth, have abandoned our inexpensive way of life and fallen into the abyss of western culture, spending money needlessly. I would like to give a small example here. When I was young, people used to bring the leaves of the mango tree (*Mangifera indica*) and the false daisy plant (*Eclipta prostrate*) and the twigs of the banyan tree (*Ficus benghalensis*), gum arabic tree (*Vachellia nilotica*) and the neem tree (*Azadirachta indica*) in the morning to brush their teeth. They did not spend any money for this, nor were they afflicted by dental diseases like today. But now, costly toothbrushes and toothpastes are

[48] The name is not clear from the Tamil text.

[49] The Sanskrit text has the word '*rās͡t͡rān͡i*'.

imported in plenty and everyone – whether young and old, poor or rich – buy and use them. Due to our fascination for western culture results, we use their toothbrush, toothpaste and other goods to our heart's content, resulting in instances of dental diseases among children and the elderly. This has led to a proliferation of dentists all over the country, who enjoy extra income due to our newfound habits. Discarding the naturally and freely available pure water, people unnecessarily spend money to buy and drink copious amounts of aerated water and other strong drinks stored in contaminated bottles. Instead of the ambrosial milk of our cows, bottles of milk from other countries have piled up even in villages, as though they are indispensable commodity. In place of the various homemade snacks, biscuits manufactured in foreign countries are bought by the cartons.

In his speech at the conference at Colombo, Sir P Ramanathan has charged that, when the British started ruling India directly since 1857, they attempted to make their language as the national language and the people tried to copy their behaviour and attire. This marked the beginning of the deterioration of the unique customs and traditions of our country and the greatness of rare disciplines like Āyurveda. Just as the English culture was the reason for the destruction of the hoary Āyurveda, our ancient habits and our country's culture too were neglected, with everything turning upside down.

If the menfolk cut their hair short due to their fascination with the modern culture, the women too, to show that they are no less than the men in any way, dared to chop off their tresses, which are considered to be sacred and which form a part of their natural beauty. Seeing this, one is forced to conclude that such dark, deluded and depraved conduct in the name of social reform will not permit even an iota of our country's hallowed virtues to remain. Yet, as long as there is even one noble person known for his venerable qualities, habits and morality, we will not surrender our ancient culture, our scriptures, especially our medical system, to the termites of modern thought.

Therefore, gentlemen, I request our Indian brothers to pledge their support for our Vedic tradition and our Āyurvedic system to whatever extent possible, to heed the salutary advice mentioned therein, to eliminate the troubles and evils caused by modern culture and rectify the situation, thereby improving our health and lifespan, and to express our total gratitude to our ancient preceptors. I also pray to Īśvara, the Refuge of the whole world, to grant them the strength to do so.

Conclusion

Gentlemen, while concluding my speech, I would like to make another important request. Around twenty years ago, I had started an organisation called '*Ten◉ɪ n◉ɪ indiya Āyurveda-Yunāni Vaiddiya Sangam*' (South Indian Āyurveda and Unāni Medical Association)

in Chennai (prior to this, there was no medical association). As its secretary, I had conducted strong propaganda and protests to develop our national medical system, which resulted in appointment of the Usman Committee by the Government. I have already mentioned that the Government had subsequently built and is running a national medical college and affiliated hospitals and that I had requested its Chief (Vaidyaratnam G. Srinivasamurti) to implement a few other reforms for its continued progress.

After I was elected as the Chief Secretary of the All India Ayurveda Mahamandal and the All India Ayurveda Vidyapeeth by their office-bearers, Providence enabled me to arrange a conference of the Mahamandal in Ceylon. In 1924, I went from here to Colombo with around 150 members, conducted the All India conference and organised publicity in many places of the island. You will be happy to know that the local representatives who had attended the conference put forward and passed a proposal in their parliament to set up an academy of Indian medicine. During the next year itself, they constructed a big college, which they are managing. When I recollected the affection shown to me by the Ceylonese and their keen desire to immediately institute the system of Āyurveda in their country and to build a college, I had to wonder how I could recompense them.

Then, from the southernmost tip, my focus shifted to the northernmost tip of India, near the Himalayas. Due to this effort, the 16th conference of the All India Ayurvedic Congress was held in Haridwar in 1925. I cannot adequately describe the encouragement shown by the people of the United Province[50]. As a result, the Government of the United Province, impelled by the representatives of the people, appointed a committed for Āyurveda and Unani; due to their recommendation, a National Medical College is being run there also.

If you do not think that I am listing these details in vain, you can surmise my intention. Nevertheless, I will explain it and then state my request. Even though I am inferior in knowledge and wealth, my endeavours in all issues concerning the public have yielded results, thanks to divine help. Especially the Sinhalese, whose religion, habits and tenets are vastly different from ours, have unhesitatingly conducted the National Medical Conference in a grand manner and made my efforts thoroughly successful.

Now, this great city of Tiruchi is like an ornament of South India. I am blessed to be here now and to render this service. Just as my attempts have been fruitful wherever I went, due to God's blessings, here too a school or hospital of Āyurveda needs to be established, for which the members of the municipalities and the local boards should decide soon and put in full efforts, thus promoting our national system of medicine and the prosperity of the nation. This is my humble request.

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[50] This corresponds approximately to the combined regions of the present states of Uttar Pradesh and Uttarakhand.

आस्तां नाम पुरातनत्वमथवा पुण्यत्वमन्यादृशं भैषज्यागमसन्ततेर्भुवि वयं भङ्गं न भर्तुं क्षमाः |
आरोग्यस्य विवृद्धिम् आमयगणप्रध्वंसमप्यादिमैः आलोच्याहितम् अस्मदीयसविधे शास्त्रं कथं शीर्यताम् ||

āstāṁ nāma purātanatvamathavā puṇyatvamanyādṛśaṁ
bhaiṣajyāgamasantaterbhuvi vayaṁ bhaṅgaṁ na bhartuṁ kṣamāḥ |
ārogyasya vivṛddhim āmayagaṇapradhvaṁsamapyādimaiḥ
ālocyāhitam asmadīyasavidhe śāstraṁ katharṁ śīryatām ||

"Let indeed the scriptures of Indian medical science be ancient or sacred or rare; we cannot bear to endure damage to this legacy in this world. The original masters of this science had made a significant contribution through investigation to establish good health as well as to destroy the group of diseases. Having considered the ill effects (of doing so), how can (we allow) such a great medical heritage fade away from amidst our own people?"

जीवातुर्जगताम् अमानुषकलेत्यर्थ्यामभिख्यां भिषग्विद्या साद्य कलेर्विलासवशतो दीनाद्दशामश्नुते |
सा पूर्वं भुवि वाग्भटादिभिषजां श्रेष्ठैस्समुत्थापिता भूयो भूतिमवाप्य वैद्यकसभा सम्भाविता जृम्भताम् ||

jīvāturjagatām amānuṣākaletyarthyāmabhikhyāṁ bhiṣag-
vidyā sādya kalervilāsavaśato dīnāddaśāmaśnute |
sā pūrvaṁ bhuvi vāgbhaṭādibhiṣajāṁ śreṣṭhaissamutthāpitā
bhūyo bhūtimavāpya vaidyakasabhā sambhāvita jṛmbhatām ||

"This divine, rare and life-saving medical science (of Āyurveda) has perhaps declined now, due to the vagaries of Kaliyuga. May the scriptures of this science – which had been raised in the past to a lofty level because of the great contributions of the well known Vāgbhaṭa and such other masters of Āyurveda – progress very well and once again attain a high position, honoured by this assembly of physicians!"